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Design and Implementation of an Evidence-Based Pilot Program to Support New Graduate Nurses' Transition to Professional Practice: Application of Soft Skills Education in an Associate Degree Program

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Submitted in partial fulfillment of the requirements

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I lift my hands and extend my deepest, most heartfelt gratitude to my Lord and Savior for being the purest source of strength in my moments of weakness. To Dr. Shirley Spencer, PhD., MSN, RN, I chose you as my capstone chair because I knew you would push me to produce a work of excellence and for this, I truly thank you! To each of the members of my Capstone committee, Dr. Sandra Karim DNP, APRN, FNP-C, CNRN and Dr. Patricia Martin, DNP, MSN, RN, thank you for your support, advice, and cheerleading. To my husband, thank you for giving me the time, space, love and support that I needed to complete this journey. To my children and granddaughter, you are the reflection of God in my life and my why. Finally, to my mom, you have been there every step of the way, praying for me and encouraging me. Each of you has made an indelible mark on my life and I could not have done any of it without you.

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Design and Implementation of an Evidence-Based Pilot Program to Support New Graduate Nurses Transition to Practice: Application of Soft Skills Education in an Associate Degree Program

Abstract

Problem Statement*:* Post-pandemic nurse graduates experienced difficulty transitioning to professional practice due to limited focus on soft skills acquisition in pre-licensure nursing curricula. ***Background:*** Faculty characterized graduates as underprepared for professional practice, stating concerns affecting retention, patient outcomes, and work satisfaction. ***Aim:*** The quality improvement project goal was to address the transition to practice readiness by providing soft skills education to associate degree students. ***Theoretical Framework:*** This project applied Benner's Novice- to-Expert Model and Duchscher's Stages of Transition. ***Methods:*** Utilizing a mixed methods approach, March-April 2024, a sample size of 28 participants completed both pre- and post-Casey-Fink Readiness for Practice Surveys (CFRPS), and ten Likert-style surveys. Two COVID-19 era graduates were also interviewed. Inclusion was limited to 2020 to 2022 graduates and current students through May 2025. ***Outcomes:*** Participants demonstrated a change in readiness for transition to practice in three of twenty areas ***Conclusion:*** This project provides a guide to provide soft skills education to improve nursing transition to practice readiness

Keywords: *COVID-19, new graduate, new nurse, novice, pandemic, readiness for practice, retention, soft skills, transition to practice, work readiness*

Design and Implementation of an Evidence-Based Pilot Program to Address New Graduate Nurse Transition to Practice: Application of Soft Skills Education in an Associate Degree Program

According to the Centers for Disease Control and Prevention (CDC, 2024), the COVID-19 pandemic resulted in 103,957,05 reported cases of infection and 1,210,929 deaths to date in the United States. During this pandemic period, which lasted from March 11, 2020- May 11, 2023, students were considerably affected. Not only was there a vast loss of life early on and residual morbidity, but there was also a significant loss to students who received education during this time. Students from elementary through college were affected due to the traditional path to learning not being an option. In addition to other college students, nursing and allied health students requiring holistic educational experiences were also adversely impacted. Scheduled clinical experiences that were meant to provide students with an opportunity to apply knowledge acquired from the academic setting were no longer available (Powers et al., 2022). These skills encompassed psychomotor (hard), cognitive and affective (soft) skills. This failure created a number of problems. The expectation that new graduate nurses would successfully transition to their new role and gain a measure of independence during the existing orientation timelines was not realized.

Undoubtedly, a successful transition to practice is the expectation of every new nurse, but predominant pandemic safety concerns altered this transition around the world. Orientation periods being extended for additional weeks were just one of the negative impacts (Powers et al., 2022). After extensions, newly licensed RNs were still unable to cope with the stressors of the role and left the discipline entirely. Those who remained learned the best way they could, but some of the things lost were not easily recovered. Incidentally, the same nurses that were

onboarded amid the pandemic and remained steadfast throughout, were trained by a variety of nursing staff. Staff that trained new nurses during this time often included agency RNs who had a variety of skill levels themselves. Currently, the academic and clinical education implications of the pandemic have been suppressed by vaccination and antiviral medications. The present atmosphere of nursing is however seeing the same pandemic era graduate RNs in the midst of training the latest wave of newly licensed RNs. This new wave of RNs also includes some two and four-year graduates who were potentially impacted by pandemic related limitations in the early months of their educational programs.

Across the world, academic institutions are once again matriculating and graduating nursing students. Lasting residual effects, such as difficult transition and lower retention, still exist. Several factors, such as those previously stated, compound the problem of filling in the psychomotor, cognitive, and affective gaps observed. Healthcare organizations and nursing schools must take pre-emptive steps to guarantee that new grads in this post-pandemic era are successfully prepared to begin their careers. Successful intervention, such as the expansion of nurse residency, extending orientation periods or pre-graduation training is crucial to restoring the current and future nursing workforce (Powers et al., 2022).

Background/Significance

In 2020, the Coronavirus caused by the SARS-CoV-2 Virus (COVID-19) was identified and determined to be a virulent airborne organism which could be spread from close human contact. The new and rapidly imposed restrictions triggered nursing programs internationally to pivot from in-person learning to a virtual format. According to Nicola et al., (2020), school closures involved over 90% of the global student population. Hospitals closed their doors to

many students for nearly a year in some cases. This change began on March 11, 2020, and was sudden, unexpected, and necessary to protect and preserve life across the globe.

Transition to practice difficulty had already been a well-documented issue globally. The contributors that intensified this problem focus were mitigations like sheltering in place and social distancing during the pandemic. Pandemic related changes created an obstacle for students' ability to gain direct clinical exposure. The changes affected the existing established nursing education model which traditionally incorporated face-to-face classes, simulation/laboratory-time, and in-person clinical experiences. This made nursing students unable to clinically practice what they learned in the academic setting. According to Powers et al., (2022), nursing programs lean heavily upon hands-on clinical experiences to solidify didactic education.

According to O'Connor (2015), hands-on experience via clinical or simulation lab experience provides for the incorporation of clinical-technical skills (hard skills development). This combined with critical thinking, problem-solving, interpersonal and interprofessional opportunities (soft skills development) would provide real-time application of both essential skills development during practicums (O'Connor, 2015). Clinical and simulation experiences also allowed the instructor the opportunity to model for students desired behavior while permitting occasions for the observation and address of concerns.

During the past four years, no generation was as deeply affected by pandemic related educational changes than Generation Z. These students spent the first portion of their college years and/or the latter years of high school socially distancing, participating in virtual classes, and internships during the height of the pandemic (Keyser, 2023). Having limited to no in-person

instruction and personal experiences led to the inadequate acquisition of "soft skills" such as critical thinking, teamwork, delegation, stress management and communication (Keyser, 2023).

During a new graduate residency group activity in 2021, a modified version of the Casey-Fink Readiness for Practice Survey (CFRPS) was given to new nurse residents. The CFRPS evaluates three areas associated with practice readiness: demographics/clinical experience, competency skill performance, and professional identity. Many of the participants were from generation Z. This is where one connection between practice readiness and soft skills deficiency was made (see Appendix A). Though hard skills acquisition continues to be nurtured throughout the entire nursing spectrum through vehicles such orientation, nurse residency, and continuing education opportunities, soft skills are often neglected. Nurturing and development of skills are equally as important as hard skills. They are essential to exercising clinical judgement, communicating well with patients, family members, physicians and other staff. Ineffective teamwork has contributed more to poor patient outcomes than the lack of any other skill. According to the Joint Commission (2017), 68.3% of avoidable adverse medical events that resulted in patient harm were caused by failures in teamwork. This issue can be directly tied to lack of communication, delegation, and efficiency needed in these skills to support teamwork. Critical thinking evolves over time, but it is expected that new graduate nurses will exhibit the most basic of this skill as it will be nurtured over time with support, training, exposure and guidance.

Problem Summary

Powers et al. (2021) reported that faculty perception of poor student preparation increased and is an included aspect of the larger problem. Authors of this article determined through Nursing Practice Readiness Tool (NPRT) that faculty felt students who learned during

the pandemic will be less prepared for professional practice upon graduation (Powers et al., 2021). This research suggests faculty distinguished pandemic era graduates as underprepared for professional practice (Powers et al., 2021), thus contributing to greater patient safety and practice preparedness worries.

There is a financial toll related to nurse turnover. As noted in Becker's Review (2024), author M. Gamble reports that the typical cost of turnover for an RN is $56,300, with the range of $45,100 to $67,500. It reports that this is an increase from the average cost of turnover for an RN in 2022, which was $52,350. The author also reported that for every percent point of change, there is a subsequent change in the RN turnover rate. As reported in the review, whether change is positive or negative, it will cost or save hospitals approximately $262,500 per year. When these figures are multiplied by new graduates, in addition to other staff lost, it can cost an organization millions of dollars (Elsevier, n.d.). Additionally, worries regarding medication errors, failure to recognize urgency and/or rescue which resulted in poor patient outcomes were also a concern (Powers et al., 2021). Increased concern existed in patient care areas that had high numbers of pandemic and post pandemic new graduate nurses (Powers et al., 2021). Compounding concerns, nursing workforce shortages were intensified by pandemic- and post pandemic nurse burnout in both experienced and new nurses (International Council of Nurses, 2021, 2022).

At a local hospital, concern was noted as an influx of new graduates who lacked basic clinical skills compounded by poor evidence of intact soft skills being hired. The organization extended orientation, promoted nurse residency, and encouraged mentoring to address deficiencies. During a new graduate residency group activity, a modified version of the Casey-Fink Readiness to Practice Survey (CFRPS) was given to nurse residents (see Appendix A). The

CFRPS evaluates three areas associated with practice readiness: demographics/clinical experience, competency skill performance, and professional identity. The result of this activity identified that new nurses in the residency program felt unprepared for professional practice. Upon further conversation with the nurse residents, they shared that they had concerns with their lack of comfort with hard skills such as placing nasogastric tubes, starting intravenous lines, and recognizing urgency with patients. They also shared that they felt uncomfortable with soft skills such as delegating tasks to ancillary staff, having difficulty managing stress and were unsure of where they fit in the team. This post-survey conversation combined with CFRPS results is where the connection between practice readiness and hard and soft skill acquisition was made.

The problem statement is that post-pandemic nurse graduates continue to experience difficulty transitioning to professional practice due to a diminished focus on soft skills acquisition in pre-licensure nursing program curricula. The development of an educational program to support the introduction of soft skills concepts will help create a bridge from nursing programs to clinical practice. This preparation may assist clinical organizations in facilitating nurse retention, safe patient outcomes, and work satisfaction. Thus, this project aims to promote competence and confidence in new graduate registered nurses through the application of soft skills. The goal is to increase the ability to effectively transition to professional practice which may increase retention and satisfaction rates for this group.

Needs Assessment

A needs assessment was conducted for the development of this project. This incorporated three components which include: pre-assessment, assessment and post-assessment. Each of the stages were performed through a collaboration of individuals from both the clinical and academic setting. The needs assessment aims to provide recommendations for evidence-based

strategies addressing new nurse graduates' difficulties in adapting to professional practice due to educational limitations during the COVID-19 pandemic. These recommendations are supplemented by an integrative review of the literature that examines this phenomenon.

Pre-Assessment

The identified gap and the driving force behind the needs assessment were highlighted by the results of the Casey-Fink Readiness for Practice Survey (CFRPS), which was administered during a nurse residency program. The CFRPS has demonstrated high internal consistency with a reliability index based on Cronbach's alpha value of 0.85. Additionally, the Press-Ganey employee engagement survey results from 2021 and 2022 were analyzed. The Press-Ganey survey is widely utilized and has strong psychometric properties, indicating its effectiveness in measuring employee engagement. Research by Deetz et al. (2020) highlights several key aspects of its reliability and validity, noting a high internal consistency with a Cronbach's alpha value ranging from 0.895 to 0.958. Furthermore, the Press-Ganey survey shows evidence of construct, convergent, discriminant, and predictive validity, supporting its efficacy as a tool for measuring employee engagement (Deetz et al., 2020).

Assessment

The needs assessment was strategic and systematically identified and prioritized the requirements of new graduates within the organization. This process informed strategic planning and resource allocation, with input from clinical and academic leaders to implement the recommendations. The evidence-based practice model utilized was the Johns Hopkins PET model. The model includes problem identification, evidence-based solution and translation to practice. In the role of the project manager, this model was utilized to address the question, what

are some evidence-based strategies to support new nurse graduates' difficulty adapting to professional practice after education limitations during the COVID-19 pandemic? Evidence such as Pass/fail rates on state boards, and Press-Ganey results were examined. Translation addressed the viability of change, and the plan devised to achieve it.

The team consisted of contributors from both the clinical and academic sectors. Members were both leadership and staff. Roles and responsibilities were delineated for participating staff (see Table 1).

Table 1

Team Members and Roles on Needs Assessment Committee

Team Members	**Role/Responsibility Post Meeting**
Clinical Education Staff Perinatal, Med/Surg, ED/ICU Educators	Shared with the team assessment and evaluation of new hire needs during orientation
Director of Nursing Education Department and Med/Surg units	Facilitates departmental changes and bridge information leaders. Communicates changes to PCMs. Serve as decision maker with executive leadership
Patient Care Managers-	Shares day-to-day operational needs identified on units related to preceptors/new grads
Unit Preceptors	Assists in identifying unit level needs related to preceptor/new grad dynamics and areas for opportunity
Student Project Manager	Organizes team for gap analysis of needs
New Graduate Nurse (6-12mos experience)	Represents and provides perspective of the new grad to the team, provides input and insight
College Representatives (Dean and Department Chairperson)	Provide insight into the needs of the new graduate from the perspective of an academic educator / Provide updates regarding any programmatic or curriculum changes,
Sponsor VP of Clinical Excellence/Chief Nursing Officer	Decision maker, approves needs assessment recommendations, executive budget decisions and communication of findings to leadership

Note: Team roles and responsibilities defined at initial meeting to discuss ongoing need for staffing, recruitment and retention/post meeting responsibilities listed

As a student project manager, nurse retention data were collected through collaborative efforts with human resources (HR) and nursing leadership. As depicted in Table 2, the raw data provided by HR offered initial retention figures for all nurses who exited the organization from December 2020 to 2022. The needs assessment focused on the 69% of nurses who left within two years of employment. Among this group, new graduate nurses constituted more than half. Additionally, over 32% of the nurses departed for agency work, while 25% transitioned to other sites or hospitals.

Table 2

Nurse Retention Table for Hospital X from 2020-2022

December 2020 to 2022	
Total RN FTE Resignation	118.05
Left for Agency-RN	37.6
Sign-On Bonus-RN	12.6
Different Organization/UCM Tx-RN	29.8
Other/Termed-RN	32.75
<2 years' experience	82.4

Note. The retention table is a compilation of data surrounding nurse departures. It was created by a nurse leader at Hospital X to track the various reasons for nursing staff departures. Information was extracted from the larger list to create this table.

HR provided additional retention data from 2022 onward (see Figure 1). This data captured overall registered nurse hires and terminations but did not separate new graduates from experienced staff or explore reasons for leaving. The retention rate varied: 25% in 2022, 13.1% in 2023, and 34.7% in 2024. Internal sources include retention data and staff satisfaction scores, while external sources include literature from database searches, NCLEX scores, and Press-Ganey reports. An integrative review is recommended to identify cost-effective and beneficial evidence-based strategies.

Figure 1

Nurse Retention Data for Hospital X from 2022-2024

Note. A retention table developed by human resources at Hospital X for 2022-2024.

The NCLEX pass rates of a local nursing program were evaluated and compared with the Illinois state average for the years 2019-2022. According to data from the Illinois Nursing Workforce Center (2024), this program achieved a pre-pandemic NCLEX pass rate of 100% in both 2019 and 2020. However, the pass rate declined significantly during the pandemic, reaching 60% in 2021, but experienced a slight recovery to 73% in 2022 as students returned to in-person learning. The same source reports that by 2023, the pass rate further increased to 93%.

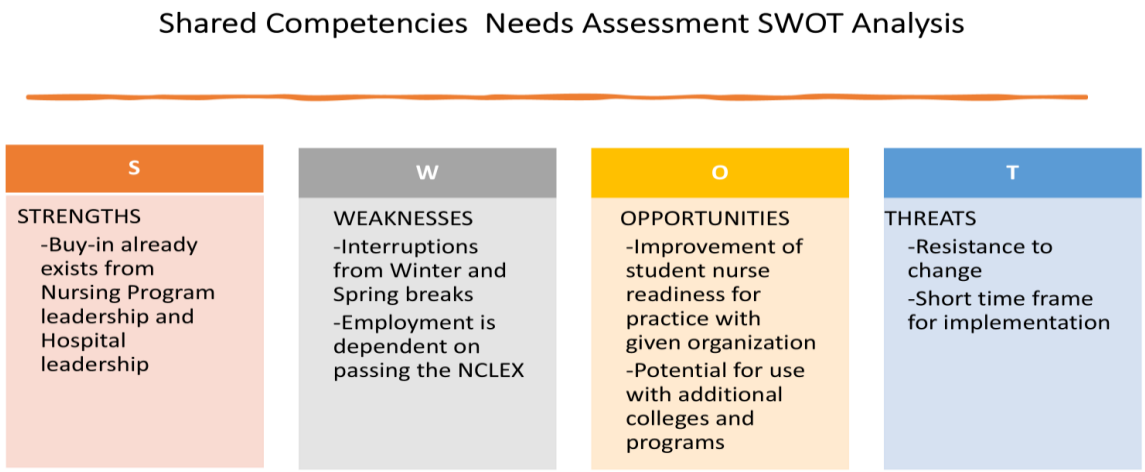
The needs assessment identified lower nurse retention and poor NCLEX pass rates as problems stemming from changes during the COVID-19 pandemic. The shift from in-person to emergency remote learning, due to stay-at-home orders, lockdowns, and social distancing, led to the closure of colleges and universities (McKay et al., 2022). Healthcare organizations also closed doors to clinical opportunities (Michel et al., 2021), affecting traditional education involving face-to-face classes, lab time, simulations, and in-person clinical experiences (McKay

et al., 2022). Research found that faculty viewed pandemic graduates as underprepared for clinical practice, with safety concerns about medication errors and failure to recognize urgent situations (Powers et al., 2021). Nursing students reported physical and psychosocial difficulties in adjusting to online learning during the pandemic (Michel et al., 2021).

As project manager, a SWOT Analysis was conducted to evaluate strengths, weaknesses, opportunities, and threats affecting the project's success. The analysis included internal and external factors of academic and clinical institutions (see Figure 2). Strengths: existing buy-in from hospital and college leadership. Weaknesses: disruption in continuity due to term breaks, students needing to pass NCLEX for employment. Opportunities: improved readiness for practice and potential model for other organizations. Threats: resistance to change and short implementation timeframe.

Figure 2

SWOT Analysis for Needs Assessment



Note. An analysis describing the strengths, weaknesses, opportunities, and threats of implementation of a process change to improve nurse practice readiness and transition to practice.

Following the SWOT analysis, the focus of the needs assessment shifted from a hospital-based residency modification to an emphasis on pre-graduate nursing students. The need for a solid foundation prior to entering the workforce was of greater concern than augmenting the existing nurse residency program. The identified causal factors contributing to the insufficient preparation of new graduate nurses included limitations in clinical experience and exposure during the pandemic. This change was reflected in the partner school's NCLEX pass rate, which decreased from 86%-100% between 2017-2020 to 60% in 2021 and 73% in 2022. According to the Illinois Nursing Workforce Center (2024), both numbers were below the state averages of 82% for 2021 and 79% for 2022 respectively.

As the project manager, a 2x2 matrix was developed for the needs assessment (see Table 2). This visual aid compares the priorities of new graduate nurses and nurse managers regarding orientation and practice readiness. The analysis highlighted differing opinions between new graduates and nursing leaders.

Table 3

2X2 Comparison for Decision Making

	High Priority for New Grads	**Low Priority for New Grads**
High Priority for Managers	Performance specific training/ Residency program re-vamp Redesign new graduate orientation	New hiring criteria Re-evaluate interview process
Low Priority for Managers	Enhanced mentoring program Frequent performance feedback protocol	New hire follow-ups

Note: Decision making process according to high and low priority for managers and new graduate nurses

Post-Assessment

The needs assessment shows that new graduates' lack of readiness for professional practice is a global issue (McKay et al., 2022; Michel et al., 2020; Powers et al., 2021). Solutions include extending orientation time, enhancing mentoring relationships, and increasing early educational opportunities. Standardizing orientation across units and evaluating new graduates' progression on an individual basis are also recommended. Findings were communicated through multiple channels. The Vice President of Clinical Excellence/Chief Nursing Officer initiated executive-level communication. Information flowed downward and upward regularly. As project manager, findings were shared between the community hospital and collegiate leaders via Zoom, in-person meetings, and email updates.

Integrative Literature Review

The purpose of this review is to add to the understanding that additional effort must be made to ensure that new graduate nurses affected by the residual effects of the pandemic are equipped to successfully transition to practice. Transition to practice programs, mentorship, preceptor training and other methods have been implemented with some success. Though several integrative reviews examine nurse readiness for practice, very little literature was available that examined the effect of the COVID-19 pandemic on this phenomenon. Limited information was available on methods that could be employed to minimize the effects of pandemic changes in the future. Past research has been performed to determine the effects of different strategies to address new graduate nurses’ readiness for practice.

The integrative literature search included CINAHL, Google Scholar, JBI Database, Medline, OVID, PubMed, Science Direct, and the OaIster database for grey literature. Search

terms used were "new graduate nurse" AND "readiness," "new graduate" AND "residency," "new nurse graduate" AND "residency" AND "pandemic," and "new graduate" AND "COVID-19." As shown in Table 3, the search was limited to peer-reviewed English articles with abstracts published from 2017-2022 (See Appendix B). Included were primary research articles from academic and nursing journals involving nursing students, new graduates, or faculty, evaluating intervention strategies for pandemic-related readiness issues. Excluded were non-English, editorial, opinion pieces, non-peer-reviewed studies, and non-primary research except systematic reviews. Articles prior to 2017 were also excluded to ensure academic rigor.

Based upon the timing of the project in respect to the onset of COVID-19, articles were categorized as pre-COVID-19 studies, studies concurrent with COVID-19, and post- COVID-19 studies. Studies with research prior to the pandemic were utilized for the value of determining what was previously known regarding strategies to improve nurse readiness for practice. Seventeen of the twenty-four articles reviewed met the inclusion criteria. Studies from 2017-2022 were included if there was relevance, closely preceding the pandemic, to nursing programs' implementation of transition to practice methods.

Table 4

Inclusion and Exclusion Criteria

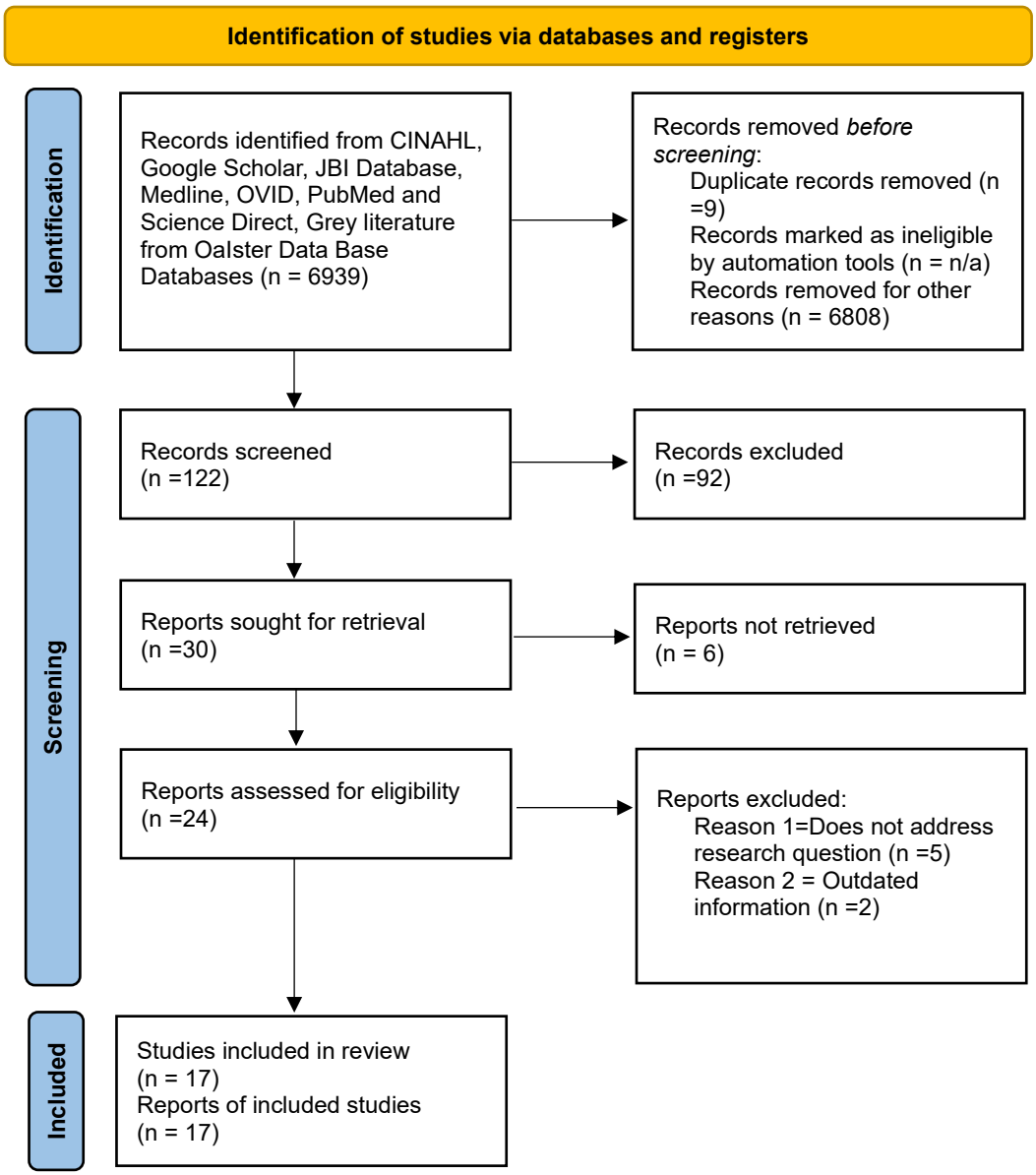
Inclusion Criteria	***Exclusion Criteria***
Articles published between 2017-2022	Publications before 2017
English language publications	Non-English language publications
Academic Journals, Grey literature	Non-scholarly articles, i.e. editorials, opinions
Primary research, including systematic reviews, integrative reviews, randomized control trials, quasi-randomized control trials, descriptive cross-sectional studies, convenience sampling, feasibility studies, questionnaire-based studies, and qualitative descriptive studies	Non-primary research inclusive of literature reviews, unless a systemic review
Peer-reviewed studies	Non-peer reviewed studies
Studies involving nursing students, nurses, or nurse faculty	Studies including non-nursing disciplines

Note. Inclusion and exclusion search criteria for integrative literature review

To further address the problem statement, several advanced searches yielded approximately 6939 results. A PRISMA model was utilized to assist in the documenting of this search (see Figure 3). The search criteria were narrowed in the larger yielding databases such as Google Scholar and Science Direct to further isolate articles that were highly relevant. After reviewing 122 article abstracts, ninety-two were rejected, thirty were sought for retrieval, but twenty-four were assessed for eligibility criteria. Upon review, seven publications were eliminated for not answering the research question or containing information that was no longer relevant. Article details were abstracted utilizing manual data abstraction methods to identify pertinent data and to generate a report that delineates the author/publication year, focus, sample/population/setting, design/method, instrument/psychometrics, key findings/results, grading quality and grade recommendation (See Appendix D).

Figure 3

PRISMA Flow Diagram for Identification of Studies via databases and registers



Note. The PRISMA model shows the number of articles found during the first article search related to COVID-19 and readiness for practice among nursing students. Source: Page MJ, et al. BMJ 2021;372: n71. Doi: 10.1136/bmj. n71. The PRISMA 2020 flow diagram templates are distributed in accordance with the terms of the Creative Commons Attribution (CC BY 4.0) license, which permits others to distribute, remix, adapt and build upon this work, for commercial use, provided the original work is properly cited. To view a copy of this license, visit https://creativecommons.org/licenses/by/4.0/

Level of Strength and Grading of Evidence

The Johns Hopkins Evidence-based Practice Model for Nursing and Healthcare professionals article appraisal workflow was utilized to validate levels of evidence between quantitative, qualitative, and mixed methods study designs (see Figure 4). Quantitative, qualitative, and mixed method studies were assessed and rated according to the five levels of evidence. Quality was assessed using the tool's three recommendations for quality. Based upon these classifications, the next step in the process was to determine the level and quality of each study. The quantitative appraisal further determined if the quantitative study was either Level I, II or III. Qualitative and mixed methods assessment criteria are also outlined in the Johns Hopkins Appraisal workflows. For these studies there were no universally established criteria to determine quality. The assessment is subjective and evaluates the degree to which a study contributes to the body of knowledge. The appraisal categories are A/B High/Good quality and C- Low quality After determining the levels of evidence an additional determination was made of the quality of evidence utilizing considerations in the appraisal tool. When applying the considerations, the quality rating A-High quality, B-Good quality, and C-Low quality were assigned to reviewed studies.

Figure 4

Johns Hopkins EBP Evidence Rating Scales

STREGNTH OF THE EVIDENCE		
Level I		Experimental study/randomized controlled trial (RCT), Systematic review of RCT with or without meta-analysis
Level II		Quasi-experimental study: including systematic review of combinations of RCT and quasi-experimental studies
Level III		Non-experimental study, qualitative study or systematic review with or without or meta-synthesis.
Level IV		Opinion of respected authorities and/or nationally recognized expert committees/consensus panels based on scientific evidence. Includes: Clinical practice guidelines and Consensus panels
Level V		Based on experiential and non-research evidence. Includes: Literature Reviews, Quality improvement, program, or financial evaluation, Case reports, Opinion of nationally recognized expert(s) based on experiential evidence
QUALITY OF THE EVIDENCE		
A High	Research	Consistent results with sufficient sample size, adequate control, and definitive conclusions; consistent recommendations based on extensive literature review that include thoughtful reference to scientific evidence.
	Summative reviews	Well-defined, reproducible search strategies; consistent results with sufficient numbers of well-defined studies; criteria-based evaluation of overall scientific strength and quality of included studies; definitive conclusions.
	Organizational	Well-defined methods using a rigorous approach; consistent results with sufficient sample size; use of reliable **and** valid measures
	Expert Opinion	Expertise is clearly evident
B Good	Research	Reasonably consistent results, sufficient sample size, some control, with fairly definitive conclusions; reasonably consistent recommendations based on fairly comprehensive literature review that includes some reference to scientific evidence
	Summative reviews	Reasonably thorough and appropriate search; reasonably consistent results with sufficient numbers of well-defined studies; evaluation of strengths and limitations of included studies; fairly definitive conclusions.
	Organizational	Well-defined methods; reasonably consistent results with sufficient numbers; use of **reliable and valid** measures; reasonably consistent recommendations
	Expert Opinion	Expertise appears to be credible.
C Low quality or major flaws	Research	Little evidence with inconsistent results, insufficient sample size, conclusions cannot be drawn
	Summative reviews	Undefined, poorly defined, or limited search strategies; insufficient evidence with inconsistent results; conclusions cannot be drawn
	Organizational	Undefined, **or** poorly defined methods; insufficient sample size; inconsistent results; undefined, poorly defined or measures that lack adequate reliability or validity
	Expert Opinion	Expertise is not discernable or is dubious.

Dang, D., Dearholt, S., Bissett, K., Ascenzi, J., & Whalen, M. (2022). *Johns Hopkins evidence-based practice for nurses and healthcare professionals: Model and guidelines.* 4th ed. Sigma Theta Tau International. © 2022 Johns Hopkins Health System/Johns Hopkins School of Nursing.

Evidence Synthesis/Appraisal

At the time of this review the number and nature of studies that resulted from applying inclusion and exclusion criteria identified 17 studies representing research from seven different countries (see Appendix C). Several articles were identified from Australia (6), the United States (5), China (3), and Nigeria (1). Some studies had no true setting as they were the result of analyses of contributions from the collective international community. The selected articles consisted of two systematic reviews with one being a quantitative review (Jarden et al., 2021) and the other a mixed methods analysis (Shatto et al., 2022). Three articles were integrative reviews (Mellor et al., 2022; Ragsdale & Schuessler 2021; Rogers et al., 2021), one scoping review (Masso et al., 2022), and one qualitative meta synthesis study (Peng et al., 2022). Additionally, three descriptive cross-sectional studies were identified and reviewed (McKitterrick et al., 2022; Nweke et al., 2021; Patterson et al., 2017). Also, one of the following: questionnaire-based study (Li et al., 2021), qualitative descriptive study (McKay et al., 2022), correlational study (Ulmen et al., 2022), Delphi study (Brooks & Mophet, 2021), convenience survey (Powers et al., 2021) quasi-experiment (Xu et al., 2021) and pre-and post-survey (Dudley et al., 2020) were reviewed. In the evidence table a review of the literature, evidence level and grade recommendations are addressed.

Several articles sought to explore what is known about the practice readiness of new graduates. These articles identified the distinguishing characteristics of the work-ready new graduate and what affects practice readiness (Brooks and Morphet., 2021; Jarden et al., 2021; Masso et al., 2022). Other articles aimed to evaluate the relationship between pre-graduate preparation and faculty/new nurse perception of readiness to practice after graduation (Dudley et al 2020; Li et al., 2021; Powers et al., 2021; Ragsdale & Schuessler, 2021). Additionally, articles that

addressed the psychosocial effect of the pandemic on graduate nurse readiness were also identified (Peng et al., 2022; Ulmen et al., 2022; Xu et al., 2021). Lastly, articles that explored strategies for evidence-based changes were also identified (Patterson et al., 2017; Rogers et al., 2021).

Theoretical Framework for Review

The integrative review of literature was guided by the synthesis of the framework elucidated by Whittemore & Knafl (2005). This framework, for the purpose of research reviews, is utilized to address concerns exclusively to the method of integrative review (Whittemore & Knafl, 2005). The initial step in this framework is to identify the problem, followed by researching literature (Whittemore & Knafl, 2005). According to the authors, the next step is an evaluation of data from primary sources and is where analyzing data and presenting the results are accomplished. To enhance the rigor of the review, data analysis methods of qualitative research are utilized by combining diverse methodologies including empirical and theoretical sources (Whittemore & Knafl, 2005). This framework increases the validity of research diversity in the initiation of evidence-based practice change.

Discussion/Evidence Themes

The successful transition from novice student nurse to new graduate nurse is essential for healthcare to maintain retention rates of new graduate nurses, promote positive patient outcomes, and work satisfaction. The financial burden of low retention and poor patient outcomes could add up to millions. According to the World Health Organization (WHO), medication errors amassed an annual cost of $42 billion, equating to 0.7% of total global health costs (Donaldson et al., 2017). This review adds to the body of evidence that transition to practice concerns is a

global problem. Several themes were identified from the integrative literature review to support this assumption, but three prevailing themes were of the greatest interest to inform the conduct of this project.

Theme #1

The first theme identified that proactive psychosocial self-support methods need to be taught prior to graduation (Mellor et al, 2022; Xu et al., 2021). The systematic review conducted by Mellor et al., (2022) noted that self-support contributes to the health and well-being of the nurse and includes spiritual, emotional, relationship, and psychological self-care. This author also shared that the capability to utilize self-care resources helps to ensure that resilience is sustained. According to Xu et al, (2021) found that group psychological counseling helped to reduce transition shock in new graduate nurses and recommends integrating this technique into transition to practice programs.

This review noted that faculty had an established concern for nursing students' level of practice readiness (McKay et al., 2022; Powers et al., 2021). As described by McKay et al., (2022), faculty shared that the shift to remote emergency clinical teaching and learning led to increased levels of stress and anxiety for both faculty and students. This negatively affected overall education delivery and receipt. Learning stress management techniques is one concept that would benefit new graduates as they encounter the changes that occur during the transition to practice.

Theme #2

The second theme was that new nurse graduates who had pre-graduation exposure working in the same organization as their professional roles were better able to enter the

discipline with a broader level of incorporated knowledge (Patterson et al, 2017; Shatto et al., 2022). According to Patterson et al., (2017) the responses of those who participated in an inventive undergraduate clinical education model, which supported the creation of optimal opportunities for students to participate in, and become familiar with, an organization and, thus, promote readiness and association with this setting. According to this author, graduates reported more readiness for practice upon completion. Shatto et al., (2022) concluded that focus on practice readiness meant educators must be capable of teaching and measuring essential activities. These activities were measured based on American Association of Colleges of Nursing (AACN) core competencies for professional nursing education domain criteria. Six of ten AACN domains were examined to evaluate competence.

This review documented that new graduate nurses had the perception that they were not adequately prepared for professional practice (Masso et al., 2022; McKitterrick et al., 2022). Both international and domestically trained students voiced a need for transparency regarding professional role expectations, direction, and support to transition to be shared earlier than when onboarding in a new position (McKitterrick et al., 2022).

Theme #3

The third theme noted was that faculty perceived and share concerns that nurse graduates are poorly prepared for practice (McKay et al., 2022; Powers et al., 2021). In the qualitative descriptive study performed by McKay et al., (2022), faculty commented that they were worried about the development of critical thinking and clinical judgement skills related to the lack of clinical exposure. According to Powers et al., (2021), faculty shared that since the pandemic students were lacking proficiency in 36 entry level measured competencies. These faculty did

not share the same level of concern regarding pre-pandemic graduates as pandemic era graduates.

Powers et al., (2021), concluded that it would be beneficial to address areas of poor proficiency through transition-to-practice program augmentation. Mellor et al., (2022) also emphasized that transition-to-practice programs would be advantageous for new graduate RNs. Transition to practice preparation can begin before new graduates enter their professional roles. Both Patterson et al., (2017) and Dudley et al., (2020), shared that there should be greater emphasis on extensive pre-graduation exposure to nursing organizations to better assimilate new graduate nurses into the culture. Both studies emphasize the importance of this time while still a nursing student to better promote practice readiness.

Key Points

The review provided an understanding of how COVID-19 has affected current new graduates' readiness for practice, while also providing insights into necessary changes. Several articles identified readiness for practice as a concern even before the pandemic, with trends worsening post-pandemic (Li et al., 2021; McKay et al., 2022; McKitterrick et al., 2022). Faculty members expressed doubts regarding the preparedness of new RNs to practice safely, suggesting that programs aimed at improving competencies would benefit students (Powers et al., 2021). Additionally, multiple studies agreed that new graduate nurses themselves lack confidence in their readiness to practice (Masso et al., 2022; McKitterrick et al., 2022). Nursing students reported difficulties in effective communication and stress management. According to Brooks & Morphet (2021), effective communication, critical thinking, and teamwork are essential nursing characteristics that support a successful transition to practice. Each article or study in this review

contributed evidence supporting the implementation of the project and/or raised questions that need addressing.

Recommendations

This review provided recommendations for designing nurse residency or transition to practice programs to increase new graduate readiness for practice (Jarden et al., 2021). The limitations of this type of educational preparation, amid pandemic concerns, led to numerous new pandemic-affected graduate nurses entering their nursing careers at a disadvantage. The deficits that new graduate nurses exhibited post-pandemic may have longer lasting effects than can be presently understood. This deficiency could potentially be passed on to the next wave of new graduates.

Evidence from the review suggests that new graduate nurses require multiple interventions to assure practice readiness. Interventions include transition to practice education, new nurse residency programs, preceptor/mentor training, and greater organizational leadership involvement (Jarden et al., 2021; McKay et al., 2022; McKitterrick et al., 2022; Mellor et al., 2022; Patterson et al, 2017). While still in a nursing program or educational setting, additional steps should be taken to prepare for the transition to practice such as complex multi-patient simulation and/or increased hands-on opportunities as these are desired by nursing students (Ragsdale & Schuessler, 2021). One of the interventions identified to assist new nurses' readiness for practice was the utilization of a pre-graduate transition education program (Patterson et al, 2017).

Limitations

The limitations of this review were noted to be potential bias related to utilizing English only or those translated English published studies. The combination of multiple methodologies and study designs may contribute to limitations with academic rigor. Timing regarding the examination of the long-term effects of COVID-19 on nursing practice for new graduate nurses was also a limiting factor as the review was conducted prior to the declared end of the pandemic. Since that time more research has become available.

Implications for Practice

Understanding the breadth and depth of how the pandemic affected the traditional learning model has provided a map and model for nursing programs for the future. The implications for practice are that programs should offer courses in a hybrid format moving forward. This would allow for students and faculty to pivot and implement remote instruction when necessary. Other implications are that the transition to practice education becomes more robust on the pre-graduate side of the nursing transition gradient and will rank psychosocial adjustment as a priority.

Conclusion

This integrated literature review is a method of synthesizing published and non-published evidence and provides levels of evidence to promote change. Several studies showed that nurse practice readiness can be affected by educational limitations (Li et al., 2021; Masso et al., 2022; McKay et al., 2022; McKitterrick et al., 2022) Pre-graduation preparation by both students and faculty is an essential component to the success of the new grad nurse and it begins upon admission to the nursing program. Evidence also suggests that improvements in preceptor

training were needed as an addition to clinical site's preparation for new grads (Mellor et al., 2022). One study concluded that transition shock plays a defining role in terms of a new nurses' experience of work-life balance and emotional exhaustion (Xu et al., 2021). Additionally, coping with low staffing levels, long shifts, lack of control and compassion fatigue were also facets of the new graduates' transition that warrants intervention (Peng et al., 2022). Adequate access to physical, emotional, and spiritual resources may also assist in the efforts to address some of the inefficiencies in nurse readiness for practice. Sufficient and early preparation, positive feedback, and encouragement may also help to improve the new nurse's responsiveness to feelings of anxiety, and nervousness. A new graduate's transition requires greater focus and coordination to achieve optimal outcomes.

Project Theoretical Framework

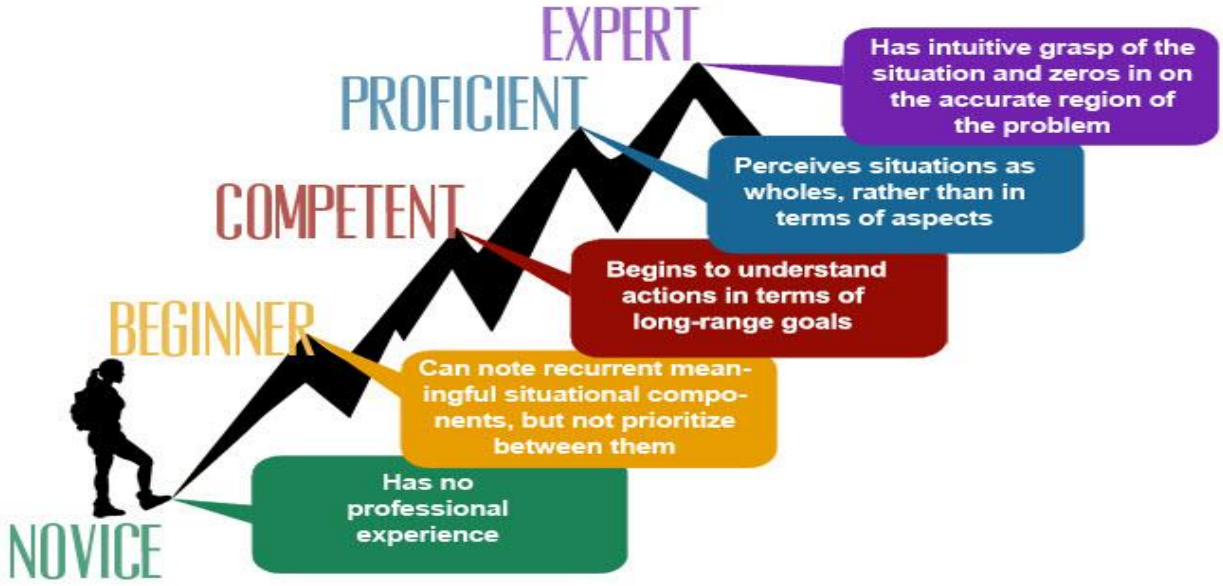
The theoretical framework applied to this project is Benner's Novice- to-Expert Model and Duchscher's Stages of Transition Theory and Transition Model. The Benner model follows a five-stage progression through levels of skill mastery from entry level nurse to one with expertise in their area of practice (see Figure 5). This model postulates that every nurse is at one of the five levels of progression as they seek to gain knowledge and mastery as they grow in this discipline. The model states that there is development from previous academic concepts to previous, tangible encounters. Each step builds upon the one prior as these theoretical tenets are developed by continued practice, and the nurse expands their clinical knowledge (Nursing Theory.org, n.d.).

For this project the focus was on the first two stages of Benner's model. Stage 1 Novice is where the nursing student in their initial year of clinical education. There are minimal opportunities to demonstrate flexibility in behavior in the novice stage. Those at this stage have

minimal skill to foresee what might occur in individual patient situations. Patient warning signs may only be obvious after a novice nurse has had practice with patients with similar signs. Stage 2 Advanced Beginner: is the new graduate nurse in their first job. These new nurses have had opportunities that have enabled them to identify frequent, important elements of a care scenario. They have some familiarity and understanding but have deficient in-depth experience. The goal of nursing leaders, educators, preceptors, etc. is to retain novice nurses and assist them in the development of skills in these first two levels to encourage the achievement of the last three levels of Benner's Model. The last three stages of Benner's model are: Stage 3 Competent, Stage 4 Proficient, and Stage 5 Expert. Beginner nurses concentrate on tasks and follow a prescribed checklist as expert nurse's attention is to the entire big picture even during the performance of tasks. The expert will notice the most subtle signs of a clinical change before a novice nurse would as this is prior to their acquisition of knowledge.

Figure 5

Benners Novice to Expert Model

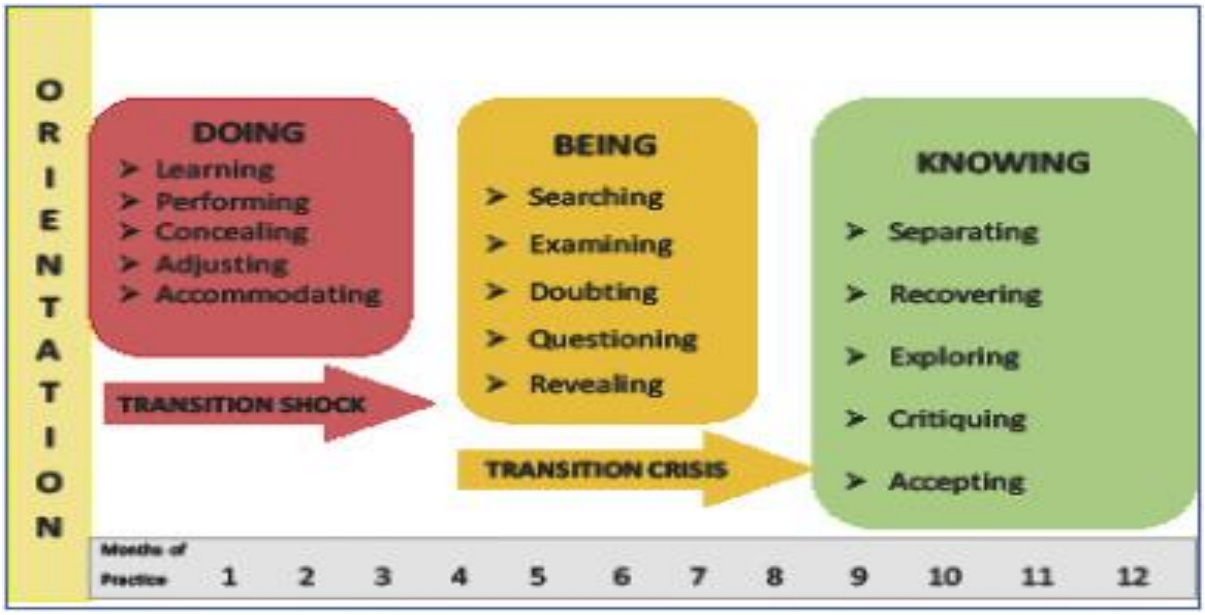


Note. Benner's Novice to Expert Model. Benner, P. (1984). From novice to expert. *Menlo Park, 84*(1480), 10-1097. Reprinted with permission.

Duchscher's Stages of Transition Theory and Transition Shock Model were also incorporated into the synthesis of literature for this review. Duchscher's model complements the principles identified in Benner's framework and provides a model of transition that can be utilized to understand the nuances of identifying difficulty in the transition of novice and beginner nurses to practice (see Figure 6). The model describes the transition as advancing through three primary stages which include doing, being, and knowing. These three phases are coexistent with the phenomenon Duchscher terms transition shock. This shock happens during the first three to four months of transition, and transition crisis, resolving near the eighth or nine months of the new graduate nurse's first year of practice (Duchscher, 2008). These models provide opportunities to examine where strategies are needed to provide transition to practice support.

Figure 6

Duchscher's Stages of Transition Theory



Note. Duchscher's stages of transition theory describe the stages that a student may experience during transition to practice. Duchscher, J. B. (2012). *From surviving to thriving: Navigating the first year of professional nursing practice*. Nursing the Future. Reprinted with permission.

According to Duchscher (2008), this model of transition evolves over 12 months and focuses on three main concepts. One of the concepts is *doing*, which includes role adjustment & teamwork. The next level shifts to *being* and is where there is greater focus on improved skill competence and critical thinking. The last concept is *knowing*, which is the development of professionalism and professional identity. Several concepts in this model are consistently intermingled with soft skills or require soft skills to progress. Communication, teamwork, critical thinking, stress management and delegation, are the five soft skills that are introduced during the 5-week project implementation.

In Duchscher's *doing* phase, the new graduate nurse is required to utilize multiple soft and hard skills. Communication, critical thinking, delegation, stress management and teamwork are each soft skills that will be applied in the *doing* or transition shock phase. In this vital period the new graduate could potentially transition easier if these soft skills are already present upon hire. These soft skills being present will better serve new graduate nurses in their identification, verbalization, coping, and sharing of positive or negative experiences as they move through this first phase. Next, as the new graduate moves past the initial orientation period and are on their own, they may find the need to increase the use of stress management techniques. This skill may be used while employing delegation and exercising teamwork abilities to maximize time management. This may help to avert or minimize a daily transition crisis. Then, as the new graduate nurse matures during the first year, they are expected to have developed an articulate confidence or *knowing* in their abilities. They should be able to fluidly apply each of the five soft skills and additional ones to their daily practice. Both models provided opportunities to examine where additional strategies are needed to provide additional transition to practice support.

Measurable Goals and Objectives

The integrative review offered evidence-based documentation that supports the need for a quality improvement project for associate degree nursing students who will be transitioning to professional practice. The recommendations that include measurable goals, objectives and rationales are as follows:

1. Goal: A change in confidence for the application of five soft skills by participants by the end of the project. Objectives: Provide Casey Fink Readiness for Practice Survey© 2008 pre- and post-project implementation to assess participants for any change in readiness to practice related to soft skills knowledge.

   Rationale: Participants communicating with other individuals, including patients, family members and members of the interprofessional team, demonstrate the application of communication soft skills (AACN 2.2, 2021). Participants executing clinical judgement principles founded on a broad knowledge base exhibit application of the critical thinking soft skill (AACN 1.3, 2021). Participants, implementing principles of team dynamics, including integrating team roles, to facilitate effective team functioning, display application of the delegation soft skill. Participants articulating a commitment to achieving holistic personal health and well-being display the application of stress management soft skills (AACN 10, 2021). Participants displaying the ability to leverage the roles and abilities of team members to optimize care demonstrate the application of teamwork soft skills (AACN 6, 2021).
2. Goal: Participants articulate an understanding of soft skills by the end of the project.

Objective: Weekly pre- and post- educational/ activity session short surveys will be given to assess students' interpretation of each soft skill as related to their future professional practice.

Rationale: Associating knowledge of each soft skill concept will encourage the application of each concept in the transition to practice.

3. Goal: Participants will transition to practice and articulate practice confidence by one year of hire.

   Objective: Provide the Casey Fink Graduate Nurse Experience Survey © (CFGNE) on 6- and 12- months of hire to assess project participant application of soft skills toward transition to practice and professional confidence

   Rationale: To increase transition to professional practice confidence leading to improved patient outcomes, retention and satisfaction rates (Jarden et al., 2021; McKay et al., 2022; McKitterrick et al., 2022; Mellor et al., 2022; Patterson et al., 2017)

Methodology

Project Design

The descriptive design for this project included a mixed methods approach consisting of data collection via quantitative and qualitative measures. These measures include interviews, observations and questionnaires. The qualitative design includes the performance of focus group conversations post-education sessions and interviews with past graduates. The quantitative method includes data collected from pre- and post-education short surveys developed to assess for the short-term change. The quantitative design also incorporates utilization of the Casey-Fink Nurse Readiness to Practice Survey (CFRPS) given pre- and post-project implementation. Using

this tool, adapted for use with permission of Drs. Kathy Casey and Regina Fink, baseline and post intervention data were collected and collated.

The complete CFRPS was presented to participants as a pre-test with the primary focus on the answers to the practice readiness assessment in section IV. Other survey data such as demographics were collected to detect patterns within groups. This method was used to determine whether the project interventions provided for participants affect readiness for professional transition-to-practice as determined by administration of the CFRPS as a post-implementation assessment. This approach was incorporated into this project by utilizing the quantifiable survey data, qualitative input from individual interviews, and focused group discussions.

Setting

The project was implemented at a local community college serving the Southland of Chicago in Illinois. The college provides education for more than 9,000 students from numerous multicultural backgrounds each year. Students here utilize more than 100 credit and non-credit programs to assist them in the pursuit of their educational or vocational journey. The Nursing Program, which awards an Associate Degree of Nursing (ADN) upon completion, was the focus for this project. This setting was chosen because there has been an increase in the number of graduates transitioning to employment at the nearby community hospital. Four of this program's former students were educated during the pandemic and contributed to the conversation of the benefit of improved pre-licensure preparation for transition to nursing practice.

Sample

The target population for this project were first- and second-year nursing students enrolled in the Fundamentals, Intermediate and Advanced Medical Surgical courses at the community college where the project will be implemented. The anticipated sample size was n = 50 participants. This minimum number was derived from the annual student enrollment anticipated for the freshman class in addition to the students advancing to the second year.

Statistical Measures

The project from which data were collected was accomplished, in part, through the administration of several surveys. These surveys were given both before and after the presentation of soft skills education and related activities. These surveys included the CFRPS, and ten short surveys developed for the project. The CFRPS survey was given pre- and post-project. The short surveys titled Communication, Critical Thinking, Delegation, Stress Management and Teamwork were given weekly pre- and post-education/activity sessions.

Descriptive statistics and frequency tables were utilized to summarize initial data to establish a baseline for comparison and correlation, to determine if there are trends in topics related to student perceived barriers to practice readiness, and to determine if there is any correlation in participant concerns to the soft skills the hospital also identified. All project data were analyzed using IBM Social Science Software Package (SPSS), version 29 (Armonk, NY). The test of reliability was Cronbach's Alpha. This was used to summarize the survey responses for 1) the pre- and post CFRPS and 2) the pre-and post-questionnaire results for the weekly educational topics covering communication, delegation, teamwork, critical thinking and stress management. For any test, a p-value of less than 0.05 indicated statistical significance.

Data Collection Tools

Two distinct data collection instruments were employed for this project. The first instrument was the Casey-Fink Readiness for Practice Survey, while the second consisted of several custom-developed questionnaires tailored to specific topics, designed to evaluate weekly learning outcomes both before and after educational activities. The Casey-Fink tool, originally copyrighted in 2008 by Kathy Casey and Regina Fink, has undergone recent revisions. The pre- and post-questionnaires for Communication, Critical Thinking, Delegation, Stress Management, and Teamwork were specifically developed to measure weekly learning outcomes.

Casey-Fink Readiness for Practice Survey

The CFRPS evaluates three areas associated with practice readiness: demographics/clinical experience, competency skill performance, and professional identity. Sections I, II and III include twenty-five items which evaluate school, gender, age, ethnicity, race, nursing assistant experience, previous degree, clinical hours, and clinical setting. They also examined competency, skill performance or skills/procedures that new graduates identify as the three (3) most uncomfortable procedures to perform independently. Section IV contained twenty additional items which examine Practice Readiness and were the primary focus of data collection. The Casey-Fink Nurse Readiness for Practice Survey is a reliable and valid instrument as the psychometric properties of the tool were studied (Casey et al., 2021). During the study evaluating this tool, a secondary analysis was performed for 71,919 new grad nurses who had at least 6 months of training in a residency program, through Vizient/AACN (Casey et al., 2021). According to the same study, “methods used for psychometric testing included exploratory and confirmatory factor analysis. The internal consistency reliability was assessed

using Cronbach's α, which yielded a value of 0.85. This indicates the degree to which items are interrelated as a group and the extent to which they consistently measure a particular concept.

Weekly Soft Skills Surveys

Similar to the pre- and post-assessment model for using the CFRPS, short Likert questionnaires were designed to evaluate changes towards achieving project learning outcomes. These were administered weekly, before and after education/activity sessions. The surveys, titled Pre- or Post-Education/Activity Survey: Communication, Critical Thinking, Delegation, Stress Management, and Teamwork, aimed to measure the level of agreement with confidence in identifying and applying each soft skills concept. The surveys assessed attitudes and participant perceptions of change before and after the educational activities.

Unlike the CFRPS, the reliability and validity of these Likert surveys were assessed using feedback from college faculty. Face validity evaluates how well a method or test measures the intended construct. The pre- and post-activity questionnaires were reviewed by a group of faculty members to identify any issues with the questions or survey flow. These faculty provided feedback to ensure that questions were clearly written, contextually appropriate, and unambiguous, enhancing the consistency and stability of the survey data. The 4-point Likert scale is within the optimal range for measuring reliability and validity. Utilizing a 4-point scale removes the neutral option and requires participants to form an opinion of agreement or disagreement (Taherdoost, 2019). This makes the 4-point scale useful for measuring attitudes or opinions (Taherdoost, 2019).

Protection of Human Rights

The ethical considerations for this project included the provision of informed consent and the protection of student confidentiality and identity in all survey responses. Informed consent was secured through the project invitation letter, which indicated that completion of the initial survey implies consent (see Appendix E). Participant selection was based on voluntary involvement, and at no time was there any suggestion or statement implying that participation would lead to improved grades. The Institutional Review Board (IRB) application for evaluating the "Design and Implementation of an Evidence-Based Pilot Program to Support New Graduate Nurses' Transition to Professional Practice: Application of Soft Skills Education in an Associate Degree Program" project was approved by Governors State University. Additionally, approval was granted by the community college IRB.

Confidentiality of student inputs via surveys was maintained by de-identifying all surveys containing demographic data. These surveys are kept in the nursing department by the department secretary in a locked file cabinet to which only she, the dean and student project manager have access. The student project manager maintained a blank roster for all participants and pre-populated a 4-digit number for each student to be randomly assigned in combination with four additional numbers to create a unique participant ID. This number was utilized to provide ease of correlation of submitted surveys from each participant. An additional number was generated after the invitation survey was complete

Procedure

The intervention implemented was the delivery of a soft skill education program developed to provide project participants with the introductory knowledge of five specific soft

skills. Hospital leaders identified the five top soft skills they desired new graduates to possess upon hire. Each soft skill aligns with the AACN Core Competencies for Professional Nursing Education (AACN, 2021). These skills include communication, delegation, teamwork, critical thinking, and stress management (see Table 5).

Table 5

Soft Skills and Associated AACN-Based Competency by Week

Week	Soft Skill	AACN Competency
1	Communication	Students effectively communicate with other individuals, including patients, family members and members of the interprofessional team (AACN, 2.2)
2	Critical Thinking	Students execute clinical judgement principles founded on a broad knowledge base (AACN, 1.3)
3	Teamwork	Students apply principles of team dynamics, including team roles, to facilitate effective team functioning (AACN, 6.2a)
4	Stress Management	Students articulate a commitment to achieving holistic personal health and well-being. (AACN, 10)
5	Delegation	Students display an ability to leverage roles and abilities of team members to optimize care (AACN, 6)
6	Make up or Additional Topic	All Skills and Competencies
7	All Skills Review	Students will actively apply all competencies into a single activity.

Note. Planned implementation timeframe and goals listed by weeks, topic. Each topic was aligned with the AACN core competency.

The project participation invitation letter/consent was sent via an email hyperlinked to the CFRPS© survey. The in-person and online active recruitment was done for an approximate 2-week window. Project invitation posters were also posted and included a QR code link to the project invitation. Initial study invites were posted in each nursing classroom and also emailed by the department secretary to every student member of the fundamental, intermediate, and advanced med-surg classes. The invitation/consent email served as a cover letter for the project

as it included an informed consent statement (see Appendix D). Participants were informed in writing that completion of the initial survey denotes the participant's acknowledgment of and provision of informed consent for participation in the project. Participants were provided with a unique identification number to place on all surveys throughout the project.

Time allotted for this educational activity was designated through permission and scheduling with the nursing department leadership, faculty and staff. During a staff enrichment meeting. information and details were provided to faculty and staff regarding their roles in supporting project implementation. During this meeting, it was identified that there could be a lack of consistency and access to participants due to different clinical site placements. This was not originally considered. Because of this information, the decision was made to complete all educational activities on the college campus during lecture meetings. This change helped to ensure consistency in the weekly delivery of educational content. It also ensured every participant's ability to engage in their hospital-based clinical hours equally.

The implementation, as noted in Table 5, included 5 weekly educational sessions and incorporated time for one or two additional weeks. Week six was allotted for a topic make-up day or additional topic per student request, but it was not needed for this purpose. Week six was replaced with the soft skills virtual escape room experience and week seven was omitted. Every session week consisted of a 15-20 minute didactic or case study session which introduced one of the five concepts. This was followed by a 10-20-minute small group/learner-focused activity, such as role playing and/or gamification of concepts with activities such as hula-hoop hands, trashketball (a miniature basketball game), deep breathing, meditation exercises and aromatherapy. Topics that required simulation were performed within the confines of the classroom as the lab supervisor was unavailable on several occasions.

The Pre- and Post- Education/Activity Questionnaire: Communication, Critical Thinking, Delegation, Stress Management and Teamwork were administered during the week each pre- and post each activity/session topic is presented and collected into two different envelopes (see Table 5). These envelopes were sealed in the classroom and handed over to the department secretary. After each education session participants were provided an opportunity to stay behind and participate in a focus group conversation. The weekly topic that was covered was the primary focus of each session for further Q &A, but participants were also given the opportunity to explore additional topics. Both project participants and non-participants opted to join focus sessions. For the preservation of time, in lieu of the weekly focus group session, the final week of educational activity the CFRPS was given to participants.

Four previous graduate students from the 2021/22 class who were also employed at the partnering hospital were invited in-person to participate. All four previous graduates agreed to the interview. Each person was interviewed individually and privately after giving verbal informed consent. Each interview was brief and utilized the same question base:

1. What aspect of the pandemic affected your preparation for professional practice?
2. What could have been done differently to prepare you for professional practice?
3. Describe how the transition to practice has been for you.
4. What are soft skills?
5. Do you feel like these skills were a part of your educational preparation?
6. How do you think soft skills knowledge could have informed your current practice?

The interviews were used to provide a baseline for the comparison of experience between pandemic era graduate nurses and project participant exposure to soft skills education.

An integrative review (see Appendix C) thoroughly examined multiple aspects of this clinical issue. The integrative literature review incorporated the analysis of relevant data that was obtained from a variety of written resources. Three predominant themes were identified. The first theme identified that proactive psychosocial self-support methods need to be taught prior to graduation (Mellor et al, 2022; Xu et al., 2021). The second theme was that new nurse graduates who had pre-graduation exposure working in the same organization as their professional roles were better able to enter the discipline with a broader level of incorporated knowledge (Patterson et al, 2017; Shatto et al., 2022). The third theme noted was that faculty perceived and share concerns that nurse graduates are poorly prepared for practice (McKay et al., 2022; Powers et al., 2021).

There was also consideration of differing ideas as to a solution. Potential systemic factors such as time, cost, stakeholder buy-in, population, and setting were considered. This led to a meticulously thought-out implementation project that seeks to address the problem from multiple perspectives. This aims to accomplish substantive and supportive improvement in nursing care and patient outcomes.

Resources/Cost Analysis

The assessment of fiscal and human resources for this project determined that there would be no extraneous utilization of financial assets during the training of participants. Training took place during the timeframe of the traditional workday in both the academic and/or clinical settings. The allocation of resources was minimal and consisted of what was on hand at each organization. Effective time management prevented the accumulation of incidental overtime for academic and/or hospital staff. Sending surveys via email or printing and distributing them to students, controlled the cost of using the postal system and minimized the amount of time needed

to receive and analyze data. The nominal cost for the production of printed questionnaires and small supplies was financially attainable with out-of-pocket funds. The efficacy of this program equates to financial savings for both organizations. New hires prepared to transition equates to increased retention rates and staff satisfaction. This prevents the financial losses incurred by nurse turnover which averages $56,300, with a range of $45,100 to $67,500 per nurse (Becker's Review, 2024).

Timeline

The project participant selection, pre-project CFRPS, and interviews began on March 4, 2024 (see Table 6). Educational content delivery was from March 18, 2024, to April 15, 2024, also noted in Table 6. The post-project CFRPS administration and data analysis began April 22, 2024. Ongoing data analysis occurred for the weekly pre-and post-survey data received.

Table 6

DNP Project Implementation Timeline

DNP Project Timeline

ACTIVITY	PLAN START	PLAN DURATION	ACTUAL START	ACTUAL DURATION	Period Highli PERCENT COMPLETE
Pre-Graduate Nurse Residency Program	Start Date	Days	Date	Weeks	Percent
Interview 21/22 Grads	3/4/2024	42		7	100%
Recruit Cohort for 23/24 project	3/4/2024	14		2	100%
Administer Pre-Intervention Surveys	3/4/2024	14		2	100%
Education Program Start	3/18/2024	1		0	100%
Educational Activity Content Delivery	3/18/2024	35		5	100%
Education Program End	4/15/2024	0		0	100%
Education Summative Evaluation Activity	4/22/2024	1		0	100%
Administer Post -Intervention Surveys	4/22/2024	1		0	100%
Analyze Data/ Prepare Report for Stakeholders	4/22/2024	1		0	100%
Report Shared with Stakeholders at College	5/2/2024	1		0	100%

Note: DNP Project Gantt chart demonstrating the project timeline beginning March 4, 2024, through April 22, 2024.

Results

Overview of data analysis

The data were imported into SPSS version 23 for Windows (IBM, 2015) and subsequently analyzed using its statistical tools. Descriptive statistics and frequency tables were used to summarize the survey responses for 1) the pre and post Casey-Fink Readiness for Practice Survey and 2) pre- and post- Education/Activity Questionnaires related to Communication, Critical Thinking, Delegation, Stress Management and Teamwork.

Paired t-tests (Field, 2013) and Pearson's correlation coefficient were originally proposed to determine if there was a difference in the perception of practice readiness (measured using the pre- and post CFRPS) before and then after the soft skills education project was completed; and Pearsons would be utilized to determine if there is any linearity in project variables. However, the dependent variables of the paired t-tests (i.e. the difference between each pair of the 20 5-point Likert scale items pre- and post CFRPS) were not normally distributed. Tests for normality were performed with the pre- and post-implementation data sets using the Shapiro-Wilk test. Wilcoxon signed-rank tests were used to determine if there was a difference in attitudes toward practice readiness (Field, 2013).

The Shapiro-Wilk test for normality indicates that data from the pre-implementation survey responses are not normally distributed. All except one variable (Q16) show a P-value $\leq \alpha$: 0.05, indicating that the data do not follow a normal distribution. Consequently, the normal distribution required for a paired t-test is not present in the pre-implementation sample data. Similarly, the Shapiro-Wilk test for normality reveals that data from the post-implementation survey responses are not normally distributed. All variables show a P-value $> \alpha$: 0.05, indicating

that these data also do not follow a normal distribution. Therefore, the normal distribution required for a paired t-test is not present in the post-implementation sample data. It is important to note that a P-value of less than 0.05 indicates statistical significance for any test.

Goal 1. Application of Soft Skills Knowledge

The implementation of an educational pilot program aims to change participant's confidence in applying five soft skills by the conclusion of the project. In this phase, the objective was to administer the Casey Fink Readiness for Practice Survey© 2008 both before and after the project implementation. This was done to evaluate any changes in participants' readiness to practice, specifically in relation to the application of soft skills. The aim was to demonstrate that there was an improvement in soft skills knowledge application, thereby enhancing the likelihood of a successful transition to professional practice. There was a total of 28 participants that began the project and 24 participants that completed the project (See Table 7). There were 89.3% female participants and 10.7% males (See Table 7 and Figure 7). Also noted in Table 7 and Figure 8, African Americans accounted for 78.6% of the participants, Caucasians represented 14.3% of the participants and both Hispanic and American Indian /Alaskan Natives represented 3.6% each.

Demographics

Approximately 28 participants took part in the CFRPS and pre-and-post Soft Skills education. Table 7 presents their demographic composition and Figures 7-9 provide visualization of this information. The average age of the project participant was 34.85 years (*SD* $\pm$9.18), with the youngest participant being 22 years old and the oldest participant being 61 years old. The majority of the participants were female (89.3%) and African American (78.6%). The higher

percentage of participants were second-year students (57.0%) while 43% were first-year students.

Table 7

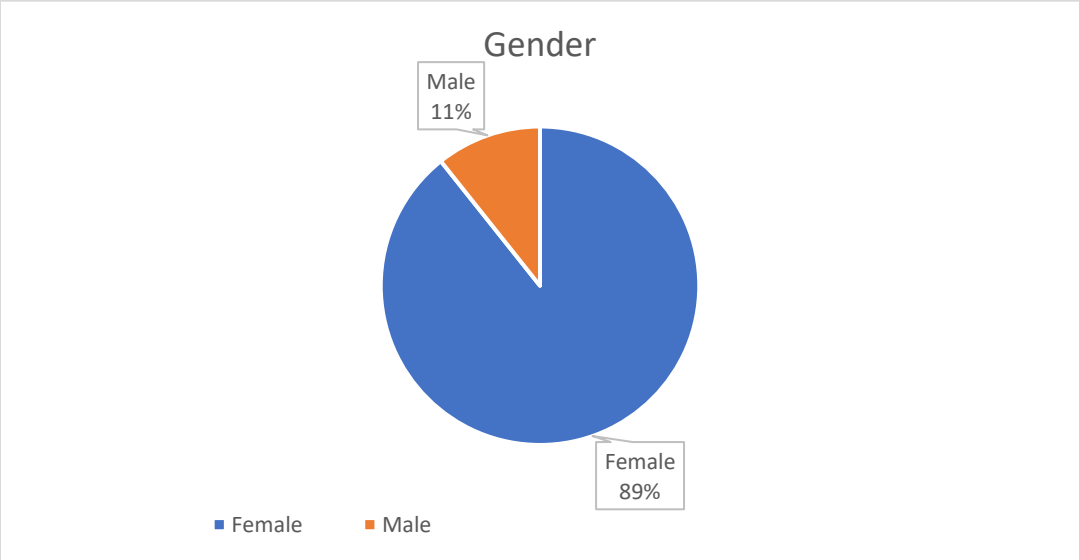
Sample Demographics

Categories	*N*	%
Age (*M* ± *SD*)	34.85 ± 9.18	
Gender		
Female	25	89.3
Male	3	10.7
Ethnicity		
American Indian or Alaskan Native	1	3.6
African American	22	78.6
Hispanic Latino	1	3.6
Caucasian	4	14.3
Year in Nursing Program		
First Year	12	43.0
Second Year	16	57.0

Note. A description of the demographic data of participants from the Soft Skills Educational intervention. *N*=the number of participants. It also displays the participants year in college at the time of the intervention.

Figure 7

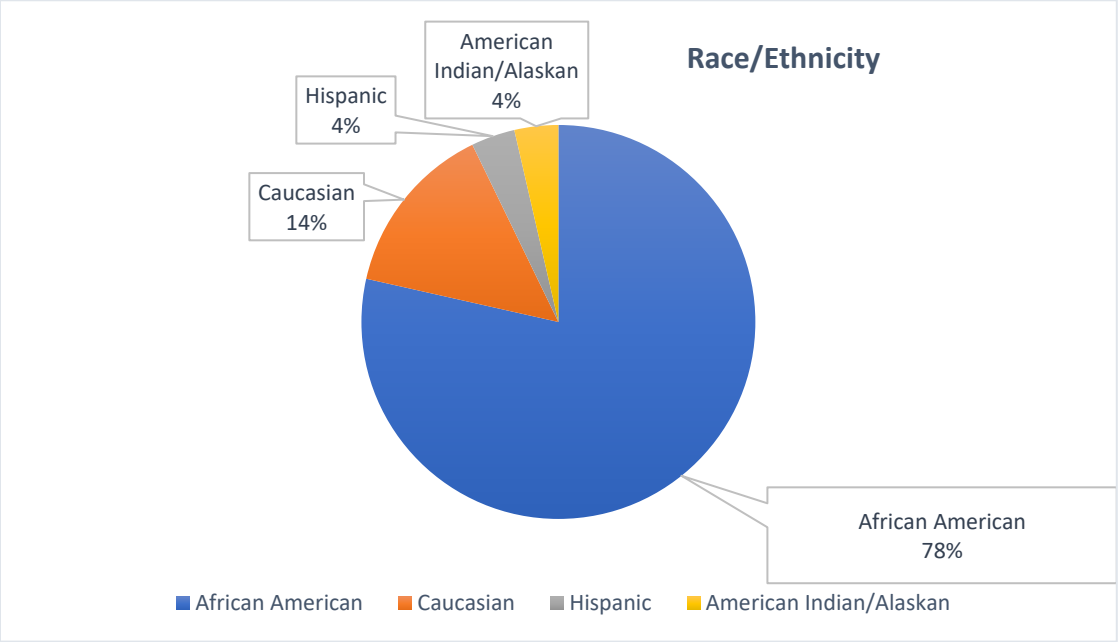
Gender-Pre-and Post-Casey Fink Readiness to Practice Survey



Note: Percentage of gender distribution for participants

Figure 8

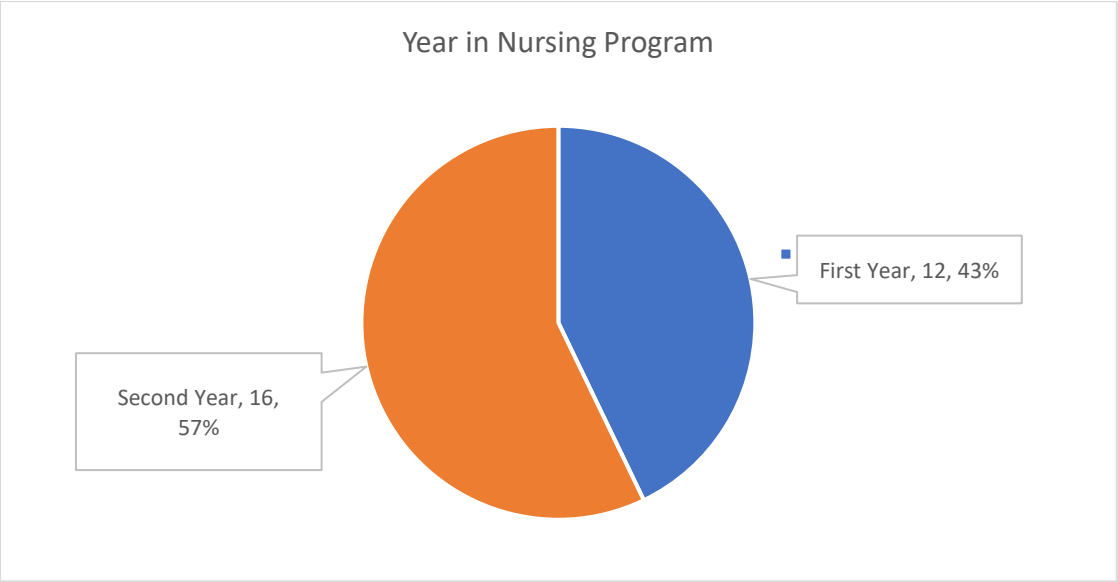
Race/Ethnicity-Pre-and Post-Casey Fink Readiness to Practice Survey



Note: Percentage of racial and ethnic make-up of participants

Figure 9

Year in Nursing Program-Pre-and Post-Casey Fink Readiness to Practice Survey



Note. Percentage of first- and second-year participants

Before the Soft Skills Education participants had a high perception of readiness to practice regarding the statement “I feel confident communicating with physicians” (Q1, *M*=3.11, Table 8). Although after the education (*M*=3.13, Table 9) participants maintained a high perception of readiness about the statement “I feel confident communicating with physicians” the difference is not statistically significant (*W*=51.00, *SE*=15.037, *z*=-1.00, *p*=.921; Table 10)

Participants reported a high perception of readiness to practice regarding the statement “I am comfortable communicating with patients from diverse populations” before (Q2, *M*=3.54, Table 8) and after (Q2, *M*= 3.63, Table 9) the soft skills education. Although after the education the participants agreed more that the perception was high about the statement “I am comfortable communicating with patients from diverse populations” than before the soft skills education, the difference is not statistically significant (*W*=56.00, *SE*=11.911, *z*=1.427, *p*=.154; Table 10)

Participants had a high perception of readiness to practice regarding the statement “I am comfortable delegating tasks to the nursing assistant” before (Q3. *M*=3.29, Table 8) and after (Q3, *M*=3.29, Table 9) soft skills education. After the education the participants agreement remained unchanged that perception was high about the statement “I am comfortable delegating tasks to the nursing assistant” the difference is not statistically significant (*W*=36.000, *SE*=11.619, z=-258, *p*=.796; Table 10)

Participants reported a low perception of readiness to practice regarding the statement “I have difficulty documenting care in the electronic medical record.” before (Q4, *M*=2.32, Table 8) and after (Q4, *M*= 1.87, Table 9) the soft skills education. Although after the education the participants reported less that the perception was low about the statement “I have difficulty documenting care in the electronic medical record.” than before the soft skills education, the difference is not statistically significant (*W*=32.500, *SE*=18.378, z=1.932 *p*=.053; Table 10)

Participants had a low perception of the of readiness to practice regarding the statement "I have difficulty prioritizing patient care needs." before (Q5, *M*=2.25, Table 8) and after (Q5, *M*=1.91, Table 9) the soft skills education. Although after the education, the participants agreed less that perception was low about the statement "I have difficulty prioritizing patient care needs" than before the soft skills education, the difference is not statistically significant (*W*=77.500, *SE*=16.804 *z*= 1.041, *p*=.298; Table 10)

Participants had a high perception of readiness to practice regarding the statement "My clinical instructor provided feedback about my readiness to assume an RN role" before (Q6, *M*=3.43, Table 8) and after (Q6, *M*= 3.04, Table 9) the soft skills education. Although there is still a high perception, after the education the participants agreed less that perception was high about the statement "My clinical instructor provided feedback about my readiness to assume an RN role" than before the soft skills education, the difference is not statistically significant (*W*=129.000, *SE*=23.825, *p*=.154; Table 10)

Participants had a high perception of readiness to practice regarding the statement "I am confident in my ability to problem solve" before (Q7, *M*=3.25, Table 8) and after (Q7, *M*=3.08, Table 9) the soft skills education. Although still a high perception, after the education the participants agreed less that perception was high about the statement "I am confident in my ability to problem solve" than before the soft skills education, the difference is not statistically significant (*W*=70.000, *SE*=14.431, *z*=1.213 *p*=.225; Table 10)

Participants had a low perception of readiness to practice regarding the statement "I feel overwhelmed by ethical issues in my patient care responsibilities" before (Q8, *M*=2.36, Table 8) and after (Q8, *M*=2.08, Table 9) the soft skills education. After the education, the participants agreed less that the perception was low about the statement "I feel overwhelmed by ethical issues

in my patient care responsibilities" than before the soft skills education. The difference is not statistically significant (*W*=65.000, *SE*=15.736, *z*=.794 *p*=.427; Table 10)

Participants had a low perception of readiness to practice regarding the statement "I have difficulty recognizing a significant change in my patient's condition" before (Q9, *M*=2.36, Table 8) and after (Q9, *M*=1.88, Table 9) the soft skills education. After the education the participants agreed less that the perception was low about the statement "I have difficulty recognizing a significant change in my patient's condition" than before the soft skills education. The difference is statistically significant (*W*=64.500, *SE*=12.309, *z*=2.072 *p*=.038; Table 10)

Participants had a low perception of readiness to practice regarding the statement "I have had opportunities to practice skills and procedures more than once." before (Q10, *M*=3.07, Table 8) and after (Q10, *M*=3.04, Table 9) the soft skills education. After the education the participants agreed less that the perception was low about the statement "I have had opportunities to practice skills and procedures more than once." than before the soft skills education. The difference is not statistically significant (*W*=64.000, *SE*=17.103, *z*=.234 *p*=.815; Table 10)

Participants had a high perception of readiness to practice regarding the statement "I am comfortable asking for help." before (Q11, *M*=3.57, Table 8) and after (Q11, *M*=3.66, Table 9) the soft skills education. After the education the participants agreed more that perception was high about the statement "I am comfortable asking for help." than before the soft skills education. The difference is not statistically significant (*W*=22.500, *SE*=7.794, *z*=.000 *p*=1.000; Table 10)

Participants had a high perception of readiness to practice regarding the statement "I use current evidence to make clinical decisions" before (Q12, *M*=3.32, Table 8) and after (Q12,

M=3.20, Table 9) the soft skills education. After the education the participants agreed less that perception was high about the statement “I use current evidence to make clinical decisions” than before the soft skills education. The difference is not statistically significant (*W*=39.500, *SE*=14.761, *z*=-.881 *p*=.378; Table 10)

Participants had a high perception of readiness to practice regarding the statement “I am comfortable communicating and coordinating care with interdisciplinary team members” before (Q13, *M*=3.29, Table 8) and after (Q13, *M*=3.10, Table 9) the soft skills education. After the education the participants agreed less that perception was high about the statement “I am comfortable communicating and coordinating care with interdisciplinary team members” than before the soft skills education, the difference is not statistically significant (*W*=30.000, *SE*=12.278, *z*=7.733 *p*=.464; Table 10)

Participants had a high perception of readiness to practice regarding the statement “Simulations have helped me feel prepared for clinical practice” before (Q14, *M*=3.18, Table 8) and after (Q14, *M*= 2.83, Table 9) the soft skills education. After the education the participants agreed less that perception was high about the statement “Simulations have helped me feel prepared for clinical practice” than before the soft skills education, the difference is not statistically significant (*W*=54.000, *SE*=22.198, *z*=1.419 *p*=.156; Table 10)

Participants had a low perception of readiness to practice regarding the statement “Writing reflective journals/logs provided insights into my own clinical decision-making skills” before (Q15, *M*=2.89, Table 8) and after (Q15, *M*=3.09, Table 9) the soft skills education. After the education the participants agreed more that the perception was low about the statement “Writing reflective journals/logs provided insights into my own clinical decision-making skills”

than before the soft skills education. The difference is statistically significant (*W*=19.500, *SE*=25.976, *z*=-3.292, *p*= <.001; Table 10)

Participants had a low perception of readiness to practice regarding the statement "I feel comfortable knowing what to do for a dying patient" before (Q16, *M*=2.96, Table 8) and after (Q16, *M*= 2.95, Table 9) the soft skills education. After the education the participants agreed less that the perception was low about the statement "I have difficulty documenting care in the electronic medical record." than before the soft skills education. The difference is not statistically significant (*W*=115.500, *SE*=25.720, *z*=.000, *p*=1.000; Table 10)

Participants had a high perception of readiness to practice regarding the statement "I am comfortable taking action to solve problems." before (Q17, *M*=3.21, Table 8) and after (Q17, *M*= 3.20, Table 9) the soft skills education. After the education the participants agreed less that perception was high about the statement "I am comfortable taking action to solve problems." than before the soft skills education. The difference is not statistically significant (*W*=56.000, *SE*=14.431, *z*=.243, *p*=.808; Table 10).

Participants had a high perception of readiness to practice regarding the statement "I feel confident identifying actual or potential safety risks to my patients" before (Q18, *M*=3.21, Table 8) and after (Q18, *M*=3.29, Table 9) the soft skills education. After the education the participants agreed more that perception was high about the statement "I feel confident identifying actual or potential safety risks to my patients" than before the soft skills education. The difference is not statistically significant (*W*=49.000, *SE*=12.619, *z*=.277, *p*=.782; Table 10).

Participants had a high perception of readiness to practice regarding the statement "I am satisfied with choosing nursing as a career" before (Q19, *M*=3.68, Table 8) and after (Q19,

M=3.54, Table 9) the soft skills education. After the education the participants agreed less that perception was high about the statement “I am satisfied with choosing nursing as a career” than before the soft skills education. The difference is statistically significant (*W*=4.500, *SE*=6.364, *z*=. -2.121, *p*=.034; Table 10).

Participants had a high perception of readiness to practice regarding the statement “I feel ready for the professional nursing role.” before (Q20, *M*=3.00, Table 8) and after (Q20, *M*=3.16, Table 9) the soft skills education. Although after education the participants agreed more that the perception was low about the statement “I feel ready for the professional nursing role” than before the soft skills education. The results are not statistically significant. (*W*=74.500, *SE*=16.545, *z*=.876, *p*=.381; Table 10).

Table 8

Summary Statistics and Participant Perception of Readiness to Practice Before Education

Pre	n	Mean	Variance	*SD*	*SE*	Median	Range	Min	Max	Perception
Q1	28	3.107	0.692	0.832	0.157	3	3	1	4	High
Q2	28	3.536	0.332	0.576	0.109	4	2	2	4	High
Q3	28	3.286	0.434	0.659	0.124	3	3	1	4	High
Q4	28	2.143	1.090	1.044	0.197	2	4	0	4	Low
Q5	28	2.071	0.958	0.979	0.185	2	4	0	4	Low
Q6	28	2.714	1.915	1.384	0.262	3	4	0	4	High
Q7	28	3.071	0.661	0.813	0.154	3	4	0	4	High
Q8	28	2.179	1.041	1.020	0.193	2	4	0	4	Low
Q9	28	2.179	0.745	0.863	0.163	2	4	0	4	Low
Q10	28	2.714	1.175	1.084	0.205	3	4	0	4	Low
Q11	28	3.571	0.476	0.690	0.130	4	3	1	4	High
Q12	28	2.964	1.073	1.036	0.196	3	4	0	4	High
Q13	28	3.107	0.766	0.875	0.165	3	4	0	4	High
Q14	28	2.643	1.275	1.129	0.213	3	4	0	4	High
Q15	28	2.714	0.804	0.897	0.169	3	4	0	4	Low
Q16	28	2.786	1.063	1.031	0.195	3	4	0	4	Low
Q17	28	3.036	0.776	0.881	0.167	3	4	0	4	High
Q18	28	3.214	0.175	0.418	0.079	3	1	3	4	High
Q19	28	3.679	0.226	0.476	0.090	4	1	3	4	High
Q20	28	3.000	0.815	0.903	0.171	3	3	1	4	Low

Note. Summary Statistics of Casey Fink Readiness to Practice Responses Prior to the Soft Skills education pilot program. The weighted average for the responses is 3.064. Aggregate Mean (μ) response below the weighted average demonstrates low perception of readiness to practice by participants for the given item number whereas, an aggregate Mean above the weighted Mean indicates a high perception of readiness to practice by participants.

Table 9

Summary Statistics and Participant Perception of Readiness to Practice After Education

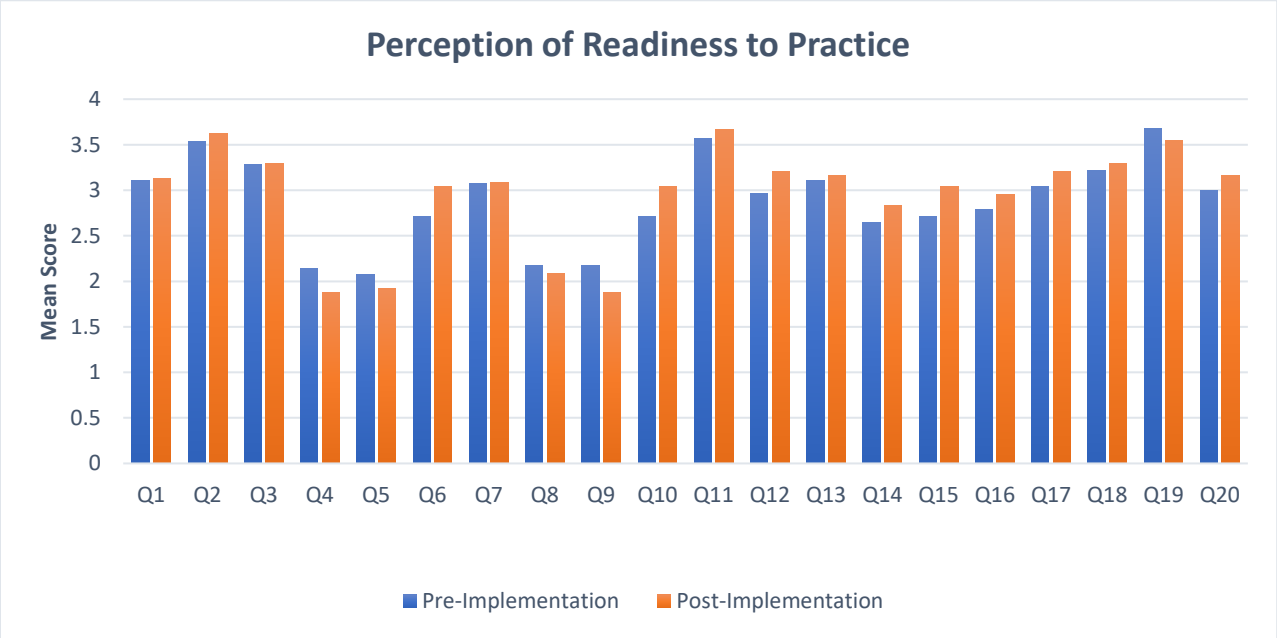
Post	n	Mean	Variance	*SD*	*SE*	Median	Range	Min	Max	Perception
Q1	24	3.125	0.549	0.741	0.151	3	3	1	4	High
Q2	24	3.625	0.332	0.576	0.118	4	2	3	5	High
Q3	24	3.292	0.303	0.550	0.112	3	2	2	4	High
Q4	24	1.875	0.549	0.741	0.151	2	2	1	3	Low
Q5	24	1.917	0.514	0.717	0.146	2	2	1	3	Low
Q6	24	3.042	0.737	0.859	0.175	3	4	1	5	High
Q7	24	3.083	0.341	0.584	0.119	3	2	2	4	High
Q8	24	2.083	0.949	0.974	0.199	2	4	1	5	Low
Q9	24	1.875	0.375	0.612	0.125	2	2	1	3	Low
Q10	24	3.042	0.476	0.690	0.141	3	3	1	4	High
Q11	24	3.667	0.232	0.482	0.098	4	1	3	4	High
Q12	24	3.208	0.172	0.415	0.085	3	1	3	4	High
Q13	24	3.167	0.406	0.637	0.130	3	2	2	4	High
Q14	24	2.833	0.754	0.868	0.177	3	3	1	4	Low
Q15	24	3.042	0.650	0.806	0.165	3	3	1	4	High
Q16	24	2.958	0.563	0.751	0.153	3	3	1	4	High
Q17	24	3.208	0.172	0.415	0.085	3	1	3	4	High
Q18	24	3.292	0.389	0.624	0.127	3	2	2	4	High
Q19	24	3.542	0.259	0.509	0.104	4	1	3	4	High
Q20	24	3.167	0.580	0.761	0.155	3	3	1	4	High

Note. Summary statistics of participant responses and perception of readiness to practice after the Soft Skills Education Pilot Program. The weighted average for the responses is 2.952. Aggregate Mean (μ) response below the weighted average demonstrates low perception of readiness to practice by participants for the given item number whereas, an aggregate Mean above the weighted Mean indicates a high perception of readiness to practice by participants

Figure 10 illustrates the mean scores of the perception of readiness to practice as recorded prior to and following the implementation of the soft skills education program.

Figure 10

Mean Score of Casey-Fink Readiness Practice Responses Before and After Soft Skills Education



Note. Mean score of perception of readiness to practice before and after project implementation by question

The comparison of attitudes toward readiness to practice before and after education is demonstrated in Table 10. Three items, (Q9, Q15 and Q19) Q9: I have difficulty recognizing a significant change in my patient's condition; Q15: Writing reflective journals/logs provided insights into my own clinical decision-making skills; Q19: I am satisfied with choosing nursing as a career, have a *p*-value less than $\geq \alpha$: 0.05, therefore these data are statistically significant. The majority of the responses are not statistically significant Q1: I feel confident communicating with physicians; Q2: I am comfortable communicating with patients from diverse populations; Q3:I am comfortable delegating tasks to the nursing assistant; Q4: I have difficulty documenting care in the electronic medical record; Q5: I have difficulty prioritizing patient care needs; Q6: My clinical instructor provided feedback about my readiness to assume an RN role; Q7: I am confident in my ability to problem solve; Q8:I feel overwhelmed by ethical issues in my patient care responsibilities; Q10: I have had opportunities to practice skills and procedures more than

once; Q11: I am comfortable asking for help; Q12: I use current evidence to make clinical decisions; Q13: I am comfortable communicating and coordinating care with interdisciplinary team members; Q14: Simulations have helped me feel prepared for clinical practice; Q16: I feel comfortable knowing what to do for a dying patient; Q17: I am comfortable taking action to solve problems; Q18: I feel confident identifying actual or potential safety risks to my patients; Q20: I feel ready for the professional nursing role.

Table 10

Comparison of Attitudes Toward Readiness to Practice Before and After Education

	Wilcoxon signed rank test				
	W	*SE*	*z*	*p*	*r*
Q1	51.000	15.037	-1.00	.921	-0.0192
Q2	56.000	11.911	1.427	.154	0.0274
Q3	36.000	11.619	-.258	.796	-0.0049
Q4	32.500	18.378	-1.932	.053	-0.0371
Q5	77.500	16.804	1.041	.298	0.0200
Q6	129.000	23.825	1.427	.154	0.0274
Q7	70.000	14.431	1.213	.225	0.0233
Q8	65.000	15.736	.794	.427	0.0152
Q9	64.500	12.309	1.072	.038	0.0206
Q10	64.000	17.103	.234	.815	0.0045
Q11	22.500	7.794	.000	1.000	0.000
Q12	39.500	14.761	-.881	.378	-0.0169
Q13	30.000	12.278	-.733	.464	-0.0140
Q14	54.000	22.198	-1.419	.156	-0.0272
Q15	19.500	25.976	-3.292	<.001	-0.0633
Q16	115.500	25.720	.000	1.000	0.000
Q17	56.000	14.431	.243	.808	0.0046
Q18	49.000	12.619	.277	.782	0.0053
Q19	4.500	6.364	-2.121	.034	-0.0407
Q20	74.500	16.545	.876	.381	0.0162

Note. The Wilcoxon signed rank test shows that data from the pre and post education survey responses are not statistically significant. *W* = Wilcoxon signed rank test statistic; *SE* = standard error; *z* = z-statistic; *p* = *p*-value; *r* = effect size, computed as $r = \frac{z}{n}$, where *n* = the number of observations over the two time points (i.e., n = 52 (= 28+24) observations for pre-test and post-

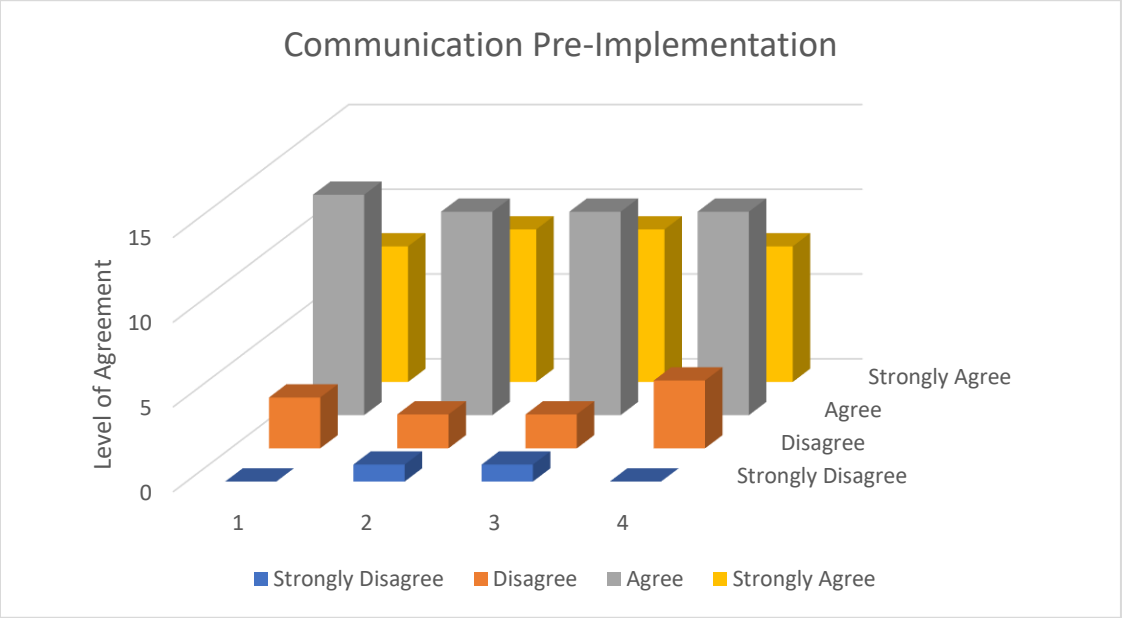
test), not the number of cases (i.e., 28) (Pallant, 2007). Using Cohen (1988) criteria for *r*, 0.1 = small effect, 0.3 = medium effect, and 0.5 = large effect.

Goal 2. Participants Articulate an Understanding of Soft Skills

The second goal was that participants would articulate an understanding of soft skills by the end of the project. Table 6 presents their demographic composition and Figures 9-11 provide visualization of this information. Figures 11-20 provide visualization of the changes in perception for each soft skill before and after the project implementation. This data was collected during the project implementation. These data are represented for the visualization of the weekly pre- and post-education changes in students' immediate perception of learning during weeks 1-5 of the project. Each of the visual representations of pre and post attitudes regarding learning demonstrates a positive change.

Figure 11

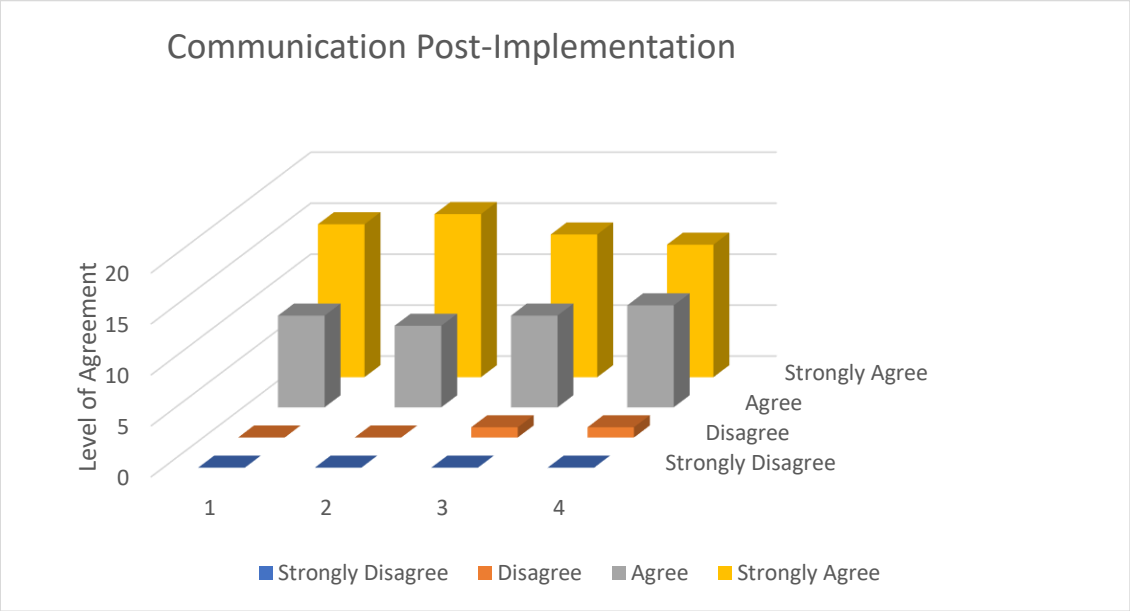
Communication Pre-Education Questionnaire Results



Note. Student responses from the Communication Pre-Education Questionnaire

Figure 12

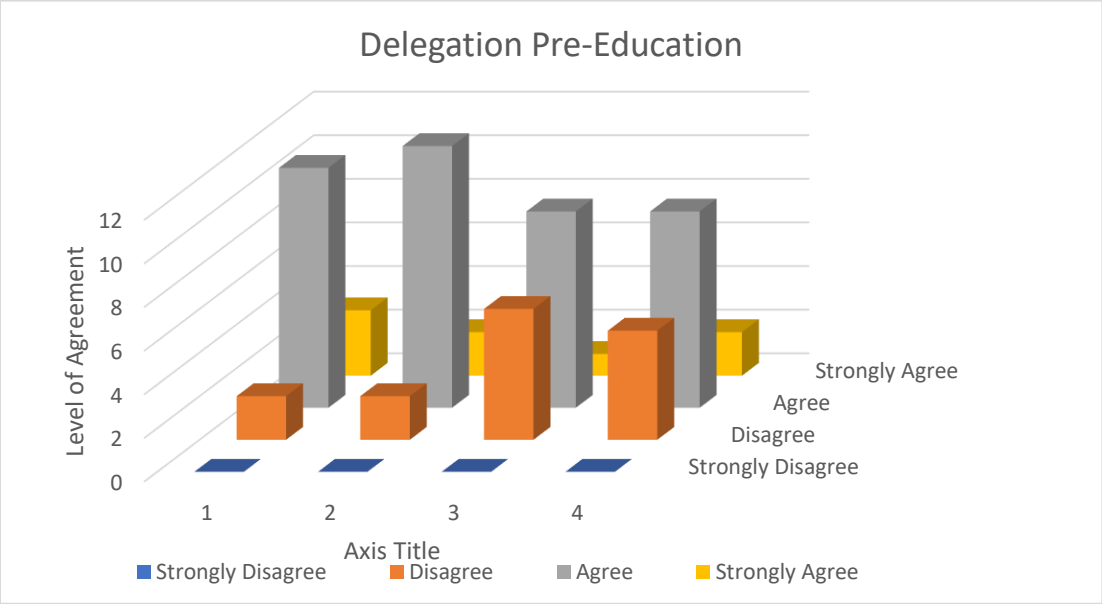
Communication Post-Education Questionnaire Results



Note. Student responses from the Communication Post-Education Questionnaire

Figure 13

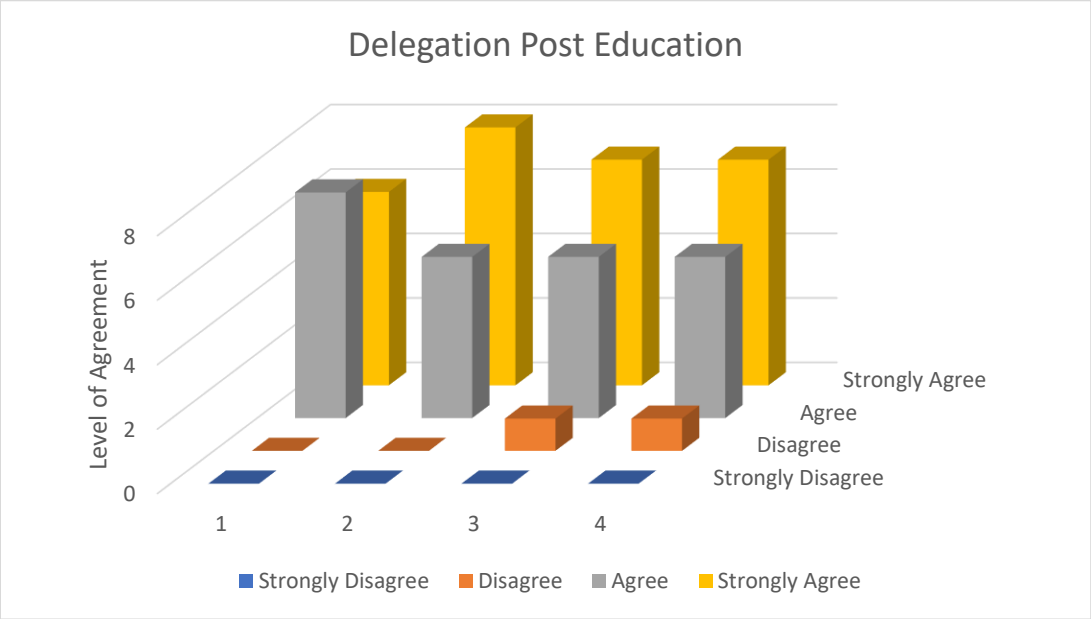
Delegation Pre-Education Questionnaire Results



Note. Student responses from the Delegation Pre-Education Questionnaire

Figure 14

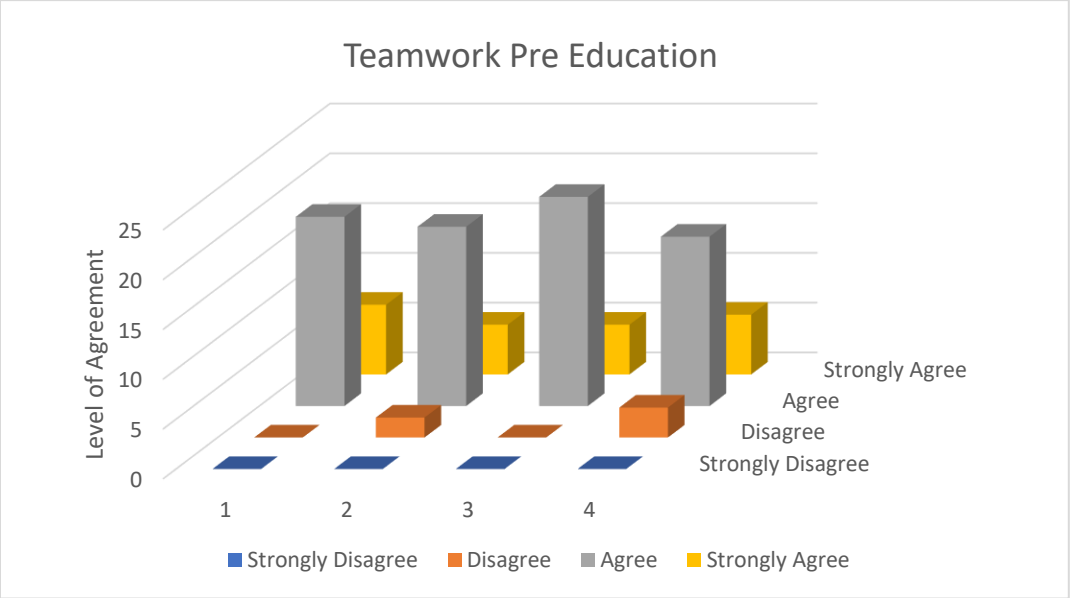
Delegation Post Education Questionnaire Results



Note. Student responses from the Delegation Post-Education Questionnaire

Figure 15

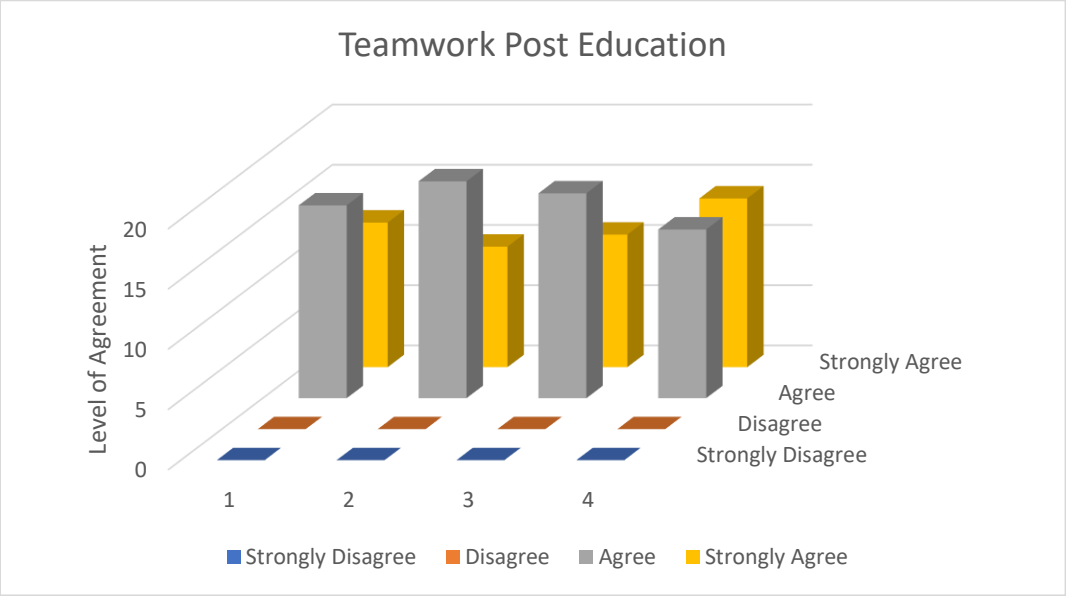
Teamwork Pre-Education Questionnaire Results



Note. Student responses from the Teamwork Pre-Education Questionnaire

Figure 16

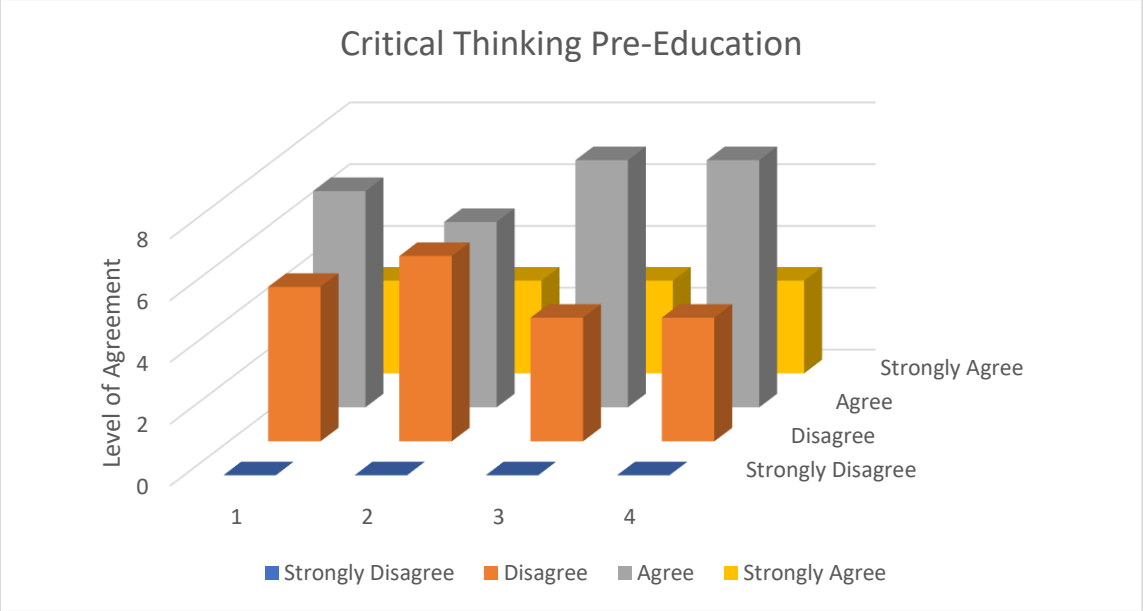
Teamwork Post Education Questionnaire Results



Note. Student responses from the Teamwork Post-Education Questionnaire

Figure 17

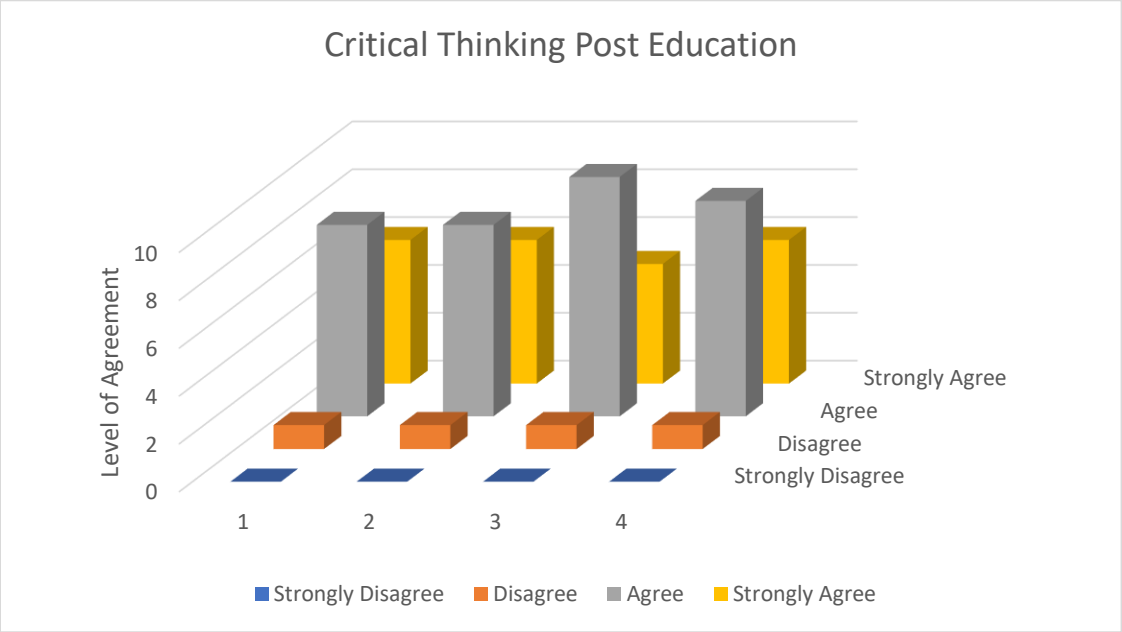
Critical Thinking Pre-Education Questionnaire Results



Note. Student responses from the Critical Thinking Pre-Education Questionnaire

Figure 18

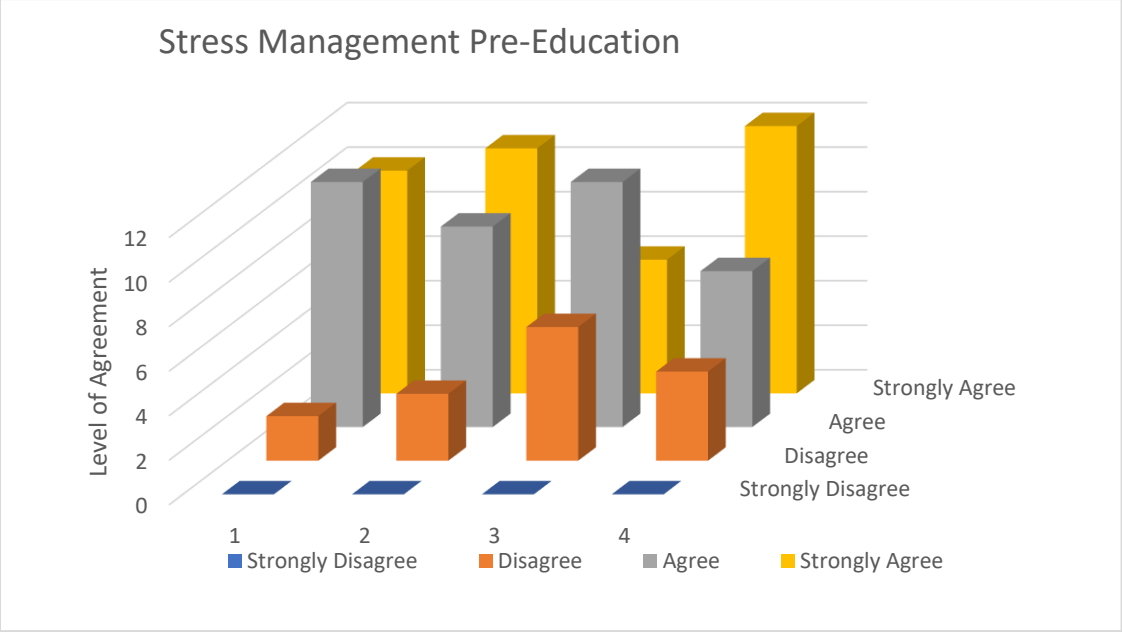
Critical Thinking Post Education Questionnaire Results



Note. Student responses from the Critical Thinking Post-Education Questionnaire

Figure 19

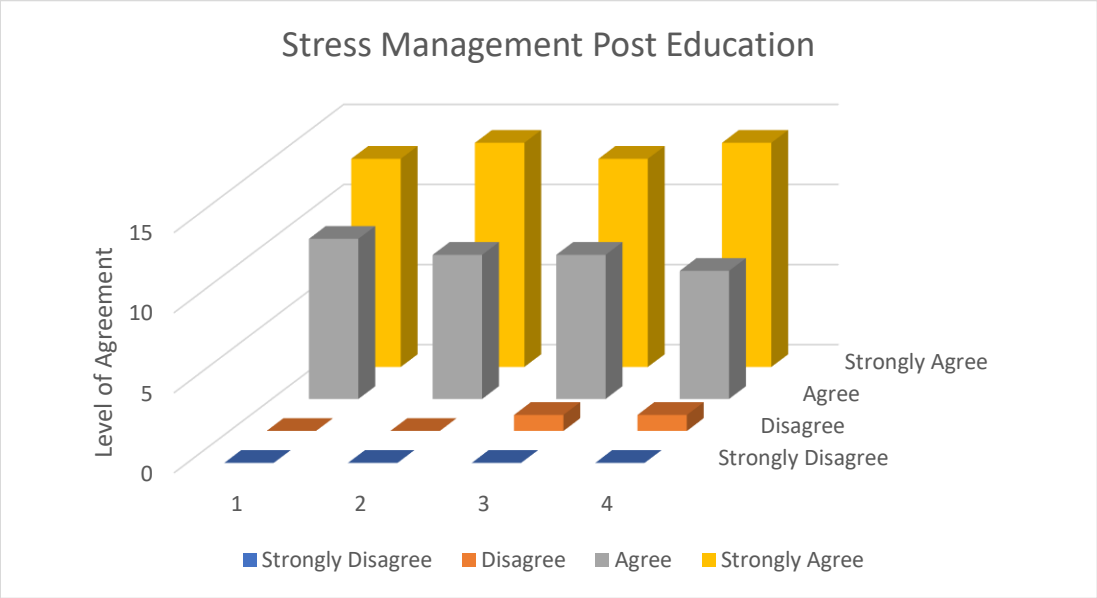
Stress Management Pre-Education Questionnaire Results



Note. Student responses from the Stress Management Pre-Education Questionnaire

Figure 20

Stress Management Post Education Questionnaire Results



Note. Student responses from the Critical Thinking Pot-Education Questionnaire

Goal 3. Participants Will Transition to Practice and Articulate Practice Confidence Within One Year of Hire.

Objective: Administer the Casey Fink Graduate Nurse Experience Survey © (CFGNE) at 6- and 12-month intervals post-hire to assess the application of soft skills in practice transitions and the development of professional confidence. At present, there is only one participant employed at the partner hospital who has not yet reached six months of employment. Consequently, this objective could not be addressed at this time. It is worth noting that the program's NCLEX pass rate for its 16 senior participants stands at 100%, with all students passing the exam within 90 days.

Evaluation

Formative

Both formative and summative evaluation were utilized for the completion of this project. Formative evaluation was performed via the data collected from pre- and post-education surveys given each weekly session throughout the project duration. This evaluation consisted of a combination of direct and indirect measures. Basic understanding of what soft skills entail and why they are desired for new graduates was the short-term goal measured. To evaluate this goal, direct measures were obtained through the administration of the weekly Soft Skills questionnaire and the analysis of those participant responses. Indirect measures were via observations and reports of change. These reports were informed through conversation with faculty who noted participants utilizing breathing techniques and inhaling aroma therapy oils, shared during the stress management presentation, prior to examinations. Notably, to maintain the focus and attention of participants, the weekly pre-education survey, education/activity, and post assessment were moved to a time slot in the middle of the participant's scheduled class time.

Summative

The logic model graphically illustrates the program's theory of change, detailing how resources, activities, and outputs are anticipated to result in desired outcomes and impact (see Appendix G). This evaluation was facilitated by the utilization of data from the pre- and post project CFRPS surveys and application of the logic model (See Appendix G). The availability of this tool throughout the project helped to inform the project while also providing a guide to move from one project phase to the next. Expected input included the use of the simulation lab and project materials, which were acquired with a budget under $100. The participation of project

members in completing requested activities was crucial for data collection. Although the plan to use the simulation lab for the project's final escape room simulation had to be altered due to staff unavailability, the activity was successfully replaced with a virtual escape room. The virtual escape room activity participants effectively demonstrated their understanding and application of soft skill concepts taught during prior weekly education sessions through an alternative output method. Apart from this modification, all planned activities were carried out as scheduled.

The anticipated outcomes were varied. The short-term objective of understanding the nature and significance of soft skills for new graduates was evaluated by comparing questionnaire results obtained before and after each weekly activity (see Tables 8-10). The mid-term outcome focused on pre-licensure nurse students' demonstration as evidenced in three areas based on the CFRPS results: recognizing significant changes in a patient's condition, writing reflective journals/logs to provide insights into clinical decision-making skills, and satisfaction with choosing nursing as a career. Each item had a p-value less than α: 0.05, indicating statistical significance. The mid-term outcome was further assessed by comparing changes in attitudes toward readiness before and after activities according to soft skill (see Table 11). The long-term outcome was not realized, as no project participant met the criteria for post-graduation participation and completing the CFGNE survey. The original plan to give the Casey-Fink Graduate Nurse Experience Survey© (CFGNE) graduate nurses who were hired as RNs by the community hospital in 2022 was also abandoned. This was due in part to the nurses no longer being considered new graduate nurses in addition to the time constraints of their shifts.

At the conclusion of the project activities, participants were asked additional evaluation questions printed on the final page of the post-CFRPS. Specifically, they were prompted to provide short responses to the questions, "What went well?" and "What could be modified?"

However, many participants did not clearly understand the directions or the intent of these questions. The instructions given to students were misinterpreted, leading many to provide responses that encompassed broad judgments about the entire nursing program rather than focusing solely on the project. After reviewing several negative responses, a recurring theme of dissatisfaction with certain program elements was noted. This data was excluded from this project's findings. Nevertheless, this information was shared with academic leadership and will be included in the academic stakeholders' report since there may be actionable insights for the nursing program leadership.

The student project manager acted as the site coordinator for each phase of the evaluation, supported by the site preceptor and additional faculty members. Contributions to project activities were aligned with the availability of faculty and/or staff resources. The effectiveness of this program was assessed based on the achievement of designated outcomes. The short-term outcome of comprehending the introduced soft skill concepts was met. Additionally, the midrange outcome of practice readiness in applying five soft skills was also accomplished. To evaluate the attainment of long-term outcomes, the Casey-Fink Graduate Nurse Experience Survey© (CFGNE) is planned to be administered to any participant agreeing to follow-up at 6-12 months post-hire. The long-term impact of the project serves as a guide for future programs to integrate soft skills education into pre-graduate training.

Findings

The results obtained from the formative data indicate that learning outcomes were achieved in the short-term goal. The results indicate students effectively learned concepts and retained their knowledge over the 6-week project. The topics Communication, Delegation, Teamwork, Critical Thinking and Stress Management were interwoven throughout the first five

weeks. Concepts introduced the first week were reiterated the second week and so on, but participants began to make the connection themselves as to how each soft skill can be built upon the other or needs the others to be more effective. The AACN Essentials Core Competency (2021), for nursing skills was the impetus for linking and presenting the soft skills in the order in which they were presented.

Participants from the second-year group demonstrated a greater level of engagement. They verbalized that they were better able to make the connection between the concepts shared during the education than what they had previously experienced as first year nursing students. The first-year group was initially less engaged in the first week, but it was noted that the level of engagement increased over the next few sessions. Second year students shared that they wished that the sessions were offered in their first year. There was sharing that there is interest in numerous other topics that fall under the heading of soft skills and hard skills. Most students, through post education conversations verbalized that they learned more from education sessions than they were previously aware as visualized in Figures 11-20.

The summative findings from the CFRPS demonstrate no statistical significance in 85% of the questions addressed. This indicates that there is currently no statistically significant change in practice readiness associated with the pre-graduate soft skills program addition. However, the formative data collected weekly demonstrates that there was a positive change in the knowledge of soft skills in participants. The formative results indicate that there is the potential to reassess and represent this project in the future. They may also be an indication that reinforcement of soft skills must be done throughout the curriculum instead of in silos

Limitations

When applying the logic model itself the inability to anticipate undesirable results that the project implementation could unintentionally precipitate was a noted limitation (See Appendix G). This was demonstrated by question misinterpretation and the subsequent responses. Limitations noted with the future implementation of soft skills education in associate degree or similar programs are the lack of buy-in from stakeholders in both the academic and healthcare sectors. Stakeholders include department leaders, in clinical and academic settings and may include students themselves. Another limitation is the lack of a specific tool designed to facilitate this type of educational program. To address these limitations, a step-by-step guide has been developed. This guide will provide stakeholders in both academic and clinical areas with the ability to assist in the development and/or implementation of soft skills education. Additionally, the future use of the Casey Fink Readiness for Practice Survey may be reconsidered to better evaluate the success or merit of soft skills education hoped to be achieved. A potential limitation could be inadequate opportunities to follow-up with new graduate nurses due to scheduling. One possibility to address this is to meet with those interested in future contact after nurse residency sessions or incorporate the survey into a new graduate nurse residency session.

Implications for Practice

The implications for nursing practice are the sharing of knowledge. They are the related opportunities within the local and global community to identify well-timed interventions for the incorporation of soft skills education into nursing program curricula. In nursing curricula, especially in community colleges, learning is highly skill based and task oriented. A greater focus on soft skills will prepare nursing students for transition to practice by sharing these skills

while students and allowing them the opportunity to grow during clinical experiences. Ultimately these students will transition from students to novice nurses with desired soft skills knowledge in place. This will also support healthcare organizations' quest to maintain patient safety outcomes, improve nurse retention and patient and staff satisfaction. This again translates to cost savings in healthcare. A deeper understanding of the need for early exposure to employer expectations for new graduate nurses is gained and will be instrumental in the improvement of the transition to nursing practice. The topic of soft skills education is farther than nursing as modifications or development of evaluative tools may promote greater accuracy in the measurement of statistical significance.

Continued exposure through other collaborative paths such as summer nurse extern opportunities, could further provide needed exposure and confidence building opportunities. This helps to indoctrinate pre-licensure students into the organizational culture and provides them occasions to explore units of interest. Additionally, there is a benefit to the partner organization in gaining needed staff while allowing them to vet the nursing student for future employment.

Dissemination

The deliverables for this project are a manuscript that qualifies for journal publication. The plan for the dissemination of the information obtained from this project was to submit a manuscript detailing the work done to *Nurse Education* for review and publication but the design of the project does not meet the standards for publication in this journal. Thus, the decision was made to pivot and submit to a publication for the Organization for Associate Degree Nursing's publication *Teaching and Learning in Nursing* that will accept the manuscript as it has been methodologically designed. The contributions to this journal are from the academic community of community colleges of nursing and the editorial board are also academic leaders. Another

deliverable is the presentation of findings to both leadership at the participating community hospital and leadership of the associate degree nursing program. The final deliverable is the development of a guide which will provide the steps for the application of pre-licensure soft skills education into nursing curricula. This guide will be utilized by the project site and potentially other academic or clinical organizations for the improvement of new graduate readiness and transition to practice education. This guide has been submitted for review.

Conclusion

Though the world is now on the other side of a global pandemic, we will forever remember how far-reaching and long-lasting some of the devastating effects were. During that period, no other member of the healthcare team was more visible than the nurse. The resilience of the nurse was placed on the stage for all to see. Some nurses stayed in the trenches, some nurses retired, some came out of retirement, some encouraged others, while others needed encouragement, but nurses were seen and appreciated. Images of nurses on the forefront and in the frontlines shifted the trajectory of nursing education and everyone wanted to be a nurse. These are those who watched from the sidelines now walking the halls of our colleges and hospital units. The pandemic has been the world's focus for the past 4 years, but the next new pandemic is on the horizon. This new pandemic is the deluge of nurse graduates that graduated from high school from 2020-2022. These new nurses missed vital socialization opportunities while they were still learning to navigate the unknown. Soft skills education is needed beyond nursing. It is needed in every discipline. It is needed in nursing more than anywhere else as nurses are typically the face of most healthcare organizations. The earlier and more consistent soft skills exposure occurs the better it will be for patients, families and the nurse themselves.

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Appendix A

Casey-Fink Readiness for Practice Survey© 2008

Kathy Casey, PhD, RN, NPD-BC and Regina Fink, PhD, APRN, AOCN, HPN, FAAN

Please fill in the blank or circle the response that represents your individual profile.

Section I - Demographics

1. **Age: _____ years**

2. **Gender:**
   a. Female
   b. Male
   c. Non-binary, transgender, or gender fluid
   d. Other (please specify) _____

3. **Ethnicity:**
   a. American Indian or Alaskan Native
   b. Asian
   c. Black or African American
   d. Hispanic or Latino
   e. Native Hawaiian or another Pacific Islander
   f. White or Caucasian
   g. 2 or more ethnicities
   h. Other (please specify) _____

4. **Other non-nursing degree (if applicable):**
   _____

5. **What previous health care work experience have you had:**
   a. Nursing assistant
   b. Medical assistant
   c. Volunteer
   d. Unit secretary
   e. EMT
   f. EMT - Paramedic
   g. Student Externship
   h. Nurse Intern or Advanced Care Partner
   i. Other: (please specify) _____

6. **Currently employed:**
   a. Yes
   b. No

7. **If yes (question #6), are you employed in a healthcare related position:**
   a. Yes
   b. No

8. **Average # hours worked/week while enrolled in Associate Degree program:** _____Hours

9. **Please share the major reasons why you chose nursing as a career.**

   _____

10. **Current GPA** _____

11. **Type of nursing program enrolled:**
    a. Traditional BSN
    b. Accelerated BSN
    a. Associate Degree
    b. Masters
    c. Other: _____

12. **Are you enrolled in an employer supported scholarship program?**
    a. Yes b. No

13. **School of Nursing attended** _____

14. **Month/year started in School of Nursing program**: _____

15. **Clinical Area of Senior Practicum (capstone) experience:**
    a. Adult M/S
    b. Adult ICU
    c. Oncology/BMT
    d. OB (L&D, POST PARTUM)
    e. Pediatric M/S
    f. Pediatric ICU
    g. NICU
    h. Mental Health
    i. Ambulatory Care Setting
    j. Rehabilitation
    k. Emergency Department
    l. OR/Perioperative Setting
    m. Other: _____

16. **Was your clinical practicum (capstone) experience at your current place of employment?**
    a. Yes b. No

17. **What setting was your clinical practicum (capstone) experience located:**
    a. Urban setting b. Rural setting

18. **How many clinical hours were you required to complete during your senior practicum?** #_____ Hours

19. **How many hours did you spend with your unit charge nurse?** #_____Hours

20. **How many primary preceptors did you have during your senior practicum experience?** #_____ Preceptors

21. **Were you required to review NCLEX-RN questions during your senior practicum course?** a. Yes b. No

22. **If yes (question 21) how many questions/weeks did you review?** # _____ Questions

23. **What did <u>YOU</u> do to prepare for your senior practicum experience: (may select more than one answer)**
    a. Practiced skills in learning lab
    b. Participated in simulation assignment
    c. Developed a care plan
    d. Brought medication reference to clinical
    e. Set daily goals with preceptor
    f. Met with preceptor prior to start of clinical experience
    g. Oriented to facility/tour unit
    h. Discussed personal learning needs with clinical faculty
    i. Did nothing to prepare
    j. Other: _____

Section II – Skills

List <u>three</u> skills/procedures you are <u>most</u> *uncomfortable performing* independently at this time? *Select from list below.*

1.

2. _____
3. _____

I am independent in all skills listed below

List of skills

Assessment skills
Bladder catheter insertion/irrigation
Blood draw/venipuncture
Blood glucose monitoring device
Central line care (dressing change, blood draws, discontinuing)
Charting/documentation
Chest tube care
EKG/Telemetry monitoring and interpretation
Giving verbal report
Intravenous (IV) medication administration
Intravenous (IV) starts
IV pumps/PCA pump operation
Medication administration
NG tube/Dobhoff care
Pulse oximetry
Responding to an emergency/CODE/changing patient condition
Trach care/suctioning
Wound care/dressing change/wound vac
Other_____

Section III – Managing Patient Care Assignment

Please answer each of the following questions by placing a mark inside the box/circle:

What is your current level of confidence in managing a patient care assignment on an adult Medical/Surgical unit:

	NOT CONFIDENT				VERY CONFIDENT
	1	2	3	4	5
Caring for 2 patients					
Caring for 3 patients					
Caring for 4 patients					

Section IV – Practice Readiness

	STRONGLY DISAGREE	DISAGREE	AGREE	STRONGLY AGREE

1. I feel confident communicating with physicians.	○	○	○	○
2. I am comfortable communicating with patients from diverse populations.	○	○	○	○
3. I am comfortable delegating tasks to the nursing assistant.	○	○	○	○
4. I have difficulty documenting care in the electronic medical record.	○	○	○	○
5. I have difficulty prioritizing patient care needs.	○	○	○	○
6. My clinical instructor provided feedback about my readiness to assume an RN role.	○	○	○	○
7. I am confident in my ability to problem solve.	○	○	○	○
8. I feel overwhelmed by ethical issues in my patient care responsibilities.	○	○	○	○
9. I have difficulty recognizing a significant change in my patient's condition.	○	○	○	○
10. I have had opportunities to practice skills and procedures more than once.	○	○	○	○
11. I am comfortable asking for help.	○	○	○	○
12. I use current evidence to make clinical decisions.	○	○	○	○
13. I am comfortable communicating and coordinating care with interdisciplinary team members.	○	○	○	○
14. Simulations have helped me feel prepared for clinical practice.	○	○	○	○
15. Writing reflective journals/logs provided insights into my own clinical decision-making skills.	○	○	○	○
16. I feel comfortable knowing what to do for a dying patient.	○	○	○	○
17. I am comfortable taking action to solve problems.	○	○	○	○
18. I feel confident identifying actual or potential safety risks to my patients.	○	○	○	○
19. I am satisfied with choosing nursing as a career.	○	○	○	○

20. I feel ready for the professional nursing role. ○ ○ ○ ○

What could be done to help you feel more prepared to enter the nursing profession?

_____

_____

Thank you for completing this survey!

Casey-Fink Permission for Use

You have our permission to use the Casey-Fink Surveys© free of cost.

Individuals are required to register for access to each survey instrument, psychometrics (reliability/validity), and scoring instructions. By registering, your information (name, title, institution where the survey is used, contact information, and purpose for use of the survey) will be kept confidential and will not be shared with third parties.

This information helps us to track usage, better understand our customers, and to ensure the surveys are conducted appropriately. After registration, you will have access to the survey materials to use in your practice or education setting. We encourage you to share your results with us so we can continue to improve the survey materials.

All Casey-Fink Surveys are copyrighted and should not be changed without written permission from the authors. Revisions made to survey items can affect the reliability and validity of the instrument.

APPENDIX B

Johns Hopkins Models and Tools Permissions

Johns Hopkins Nursing
Center for Evidence-Based Practice

Thank you for your submission.

We are happy to permit you to use the Johns Hopkins Evidence-Based Practice model and tools to adhere to our legal terms noted below.

No further permission for use is necessary.

You may not modify the model or the tools without written approval from Johns Hopkins.
All references to source forms should include "© 2022 Johns Hopkins Health System/Johns Hopkins School of Nursing."
The tools may not be used for commercial purposes without special permission.
If interested in commercial use or discussing changes to the tool, please email ijhn@jhmi.edu.

Available Downloads:

2022 JHEBP Tools (printable)

2022 EBP Tools (fillable)

APPENDIX C

Evidence Search Table

DATE SEARCHED	DATABASE	DATA TYPE	KEY SEARCH TERMS	RESULTS
10/15/2022	PubMed/Medline	Articles/ Academic Journals	"New graduate nurse readiness, COVID-19"	12
10/15/2022	PubMed/Medline	Articles/ Academic Journals	"Nurse transition to practice"	569
10/15/2022	CINAHL	Articles/ Academic Journals	"New graduate and readiness"	173
10/15/2022	CINAHL	Articles/ Academic Journals	"New graduate nurse"	2,596
10/22/2022	CINAHL	Articles/ Academic Journals	"New graduate nurse and COVID-19"	151
10/22/2022	CINAHL	Articles/ Academic Journals	"New graduate nurse, transition to practice"	550
10/22/2022	CINAHL	Articles/ Academic Journals	"Soft skills"	206
10/22/2022	EBSCO	Articles/ Academic Journals	"Soft skills and nursing"	76
10/23/2022	EBSCO	Articles/ Academic Journals	"Soft skills and nursing 2018-2023)"	48
10/23/2022	Sage Journals	Articles/ Academic Journals	new graduate nurse transition to practice and Covid 19	215
10/23/2022	Science Direct	Articles/ Academic Journals	"New graduate nurse readiness and COVID-19"	222
10/23/2022	Science Direct	Articles/ Academic Journals	"Soft skills and new graduate nurses"	364
10/30/2022	Science Direct	Articles/ Academic Journals	"Nurse residency and pandemic	864
10/30/2022	Science Direct	Articles/ Academic Journals	"Nurse graduate transition to practice and COVID-19"	834
10/30/2022	Grey Literature/OaIster	Articles/ Academic Journals	Nurse graduate and COVID-19, Nurse residency, Transition to practice	105

Appendix D

Evidence Table

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Brooks & Morphet (2021)	*N*=12 / clinicians *N*=18 / academics *N*= 220 / nursing students/ Australia	Three-round Delphi study/ Workplace readiness scale/ not listed	What are the similarities and differences in the defining characteristics of work-ready nurse graduate nurses as identified by academics, clinicians, and new graduate nurses?	Benner's novice to expert	To identify the distinguishing characteristics of the work ready new graduate	Round one data collected and round two and three groups prioritized data collected in round one	This study identified that nurse graduates should be safe practitioners with a broad range of professional conducts and including a foundational academic clinical knowledge base	V / B
Dudley et al., (2020)	*N*= 26 / fellows *N*=18 / inside organization *N*=31 / outside organization/ Australia	Pre-post survey study Clinical Learning Environment Inventory (CLEI) and Work Readiness Scale for Graduate nurses (WRS-GN) 2015/ provisional copy used as provided by the authors prior to psychometric testing	Not discussed	Not discussed	To evaluate the relationship between pre-graduate preparation and the new nurse perception of readiness to practice after graduation	Participants self-completed the CLEI and WRS-GN at the end of BSN program and again 8-10 weeks post-graduation.	The clinical learning setting only accounted for a small-to-moderate amount of nurse graduate readiness to practice	II / C

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Jarden et al., (2021)	*N*=34/ Studies/	Quantitative study: Systematic review	(1) What is the prevalence of graduate nurses' wellbeing, work wellbeing, and mental health? (2) What are the predictors of graduate nurses' wellbeing, work wellbeing, and mental health? (3) What are the barriers and enablers of graduate nurses' well- being, work wellbeing, and mental health?	The Joanna Briggs Institute method was utilized to guide this examination of studies	To examine published frequency, predictors, obstacles and facilitators of new graduate registered nurse wellbeing, work wellbeing and mental health	Quantitative studies were systematically identified, screened, and appraised alongside pre-defined inclusion criteria	Recommendation that prospective research focuses on the gap in strong evidence for the efficacy of approaches designed to assist the transition to practice of new graduate RNs. Need discovered for improved preceptor training High likelihood of intention to leave current work situation Moderate to high levels of burnout related to staffing, length of shift, workload, and compassion fatigue Limited positive feedback noted	II / B

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Li et al., (2021)	*N*=230 students *N*= 95 faculty/ China	Questionnaire-based study	None	None	To examine attitudes of students and instructors toward online education during the COVID-19 pandemic	Electronic questionnaires were utilized in both English and Chinese	Most teachers were satisfied with online instruction Most students were dissatisfied with online instruction Students who remained in the country had higher rates of satisfaction with online learning	III / B

Masso et al., (2022)	*N*=45 / reviews/Australia	Scoping review	What is known about the practice readiness of new graduate nurses upon commencement of employment and during their first year of employment? What factors influence the practice readiness of new graduate nurses during their first year of employment?	Arksey & OMalley framework	To explore what is known about practice readiness of new graduates when beginning employment, how these changes during the first year, and what affects practice readiness	A five-stage scoping review was performed	Concerns about practice readiness are not well supported by evidence. Lack of confidence in the beginning of the new graduate nursing career is well supported by evidence	V / B

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
McKay et al., (2022)	*N*=19 / baccalaureate clinical faculty/ United States	Qualitative descriptive study	none	Lewin's Change Theory	To develop a description of experiences of BSN nursing faculty transition to remote learning during the COVID-19 pandemic	Social media posts with embedded electronic survey links were posted on Facebook, LinkedIn, and Twitter inviting those who satisfied inclusion criteria to participate	Five themes emerged from data Understanding the effect on the nurse faculty can support recommendations for best practices in the future Perception of students would provide value to current findings	III / B
McKitterrick et al., (2022)	*N*=83, international nursing students/ Australia	Descriptive cross-sectional survey The Casey-Fink Readiness for Practice Survey as well as newly added questions to measure transition, clinical preparedness, facilitators, and barriers that could affect transition and the impact of Coronavirus disease of 2019 on transition. The Casey-Fink survey is both valid and reliable	1.How do international nursing students feel about their preparation to transition to practice? 2. How do international nursing students report their comfort and confidence in performing nursing skills independently? 3. What is the level of confidence to manage a full patient care assignment in an adult medical/surgical unit? and 4. What do international nursing students	Not noted	To examine international nursing students' perception of their preparation and experiences from their educational backgrounds related to entering the nursing workforce.	Survey data were collected from 26 October 2020–31 January 2021 from international nursing students in Australia. The survey was administered electronically via Research Electronic Data Capture (Redcap)	There was a 90 percent response rate. Over fifty percent reported readiness to work as nurses. Over fifty percent reported a lack of readiness to gain employment in the host country. Clinical simulation was found to be helpful in preparation for transition. Barriers included visa status, financial concerns, length of clinical placements, language barriers and knowledge of the host country's health care system.	III / B

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
			report as facilitators and barriers that affect their transition to practice?					
Mellor et al., (2022)	*N*=41 / studies	Integrative review of qualitative, quantitative, and mixed methods studies/ McGill Mixed methods Appraisal Tool	1. Are there clear research questions? 2. Do the data collected allow us to address the research questions?	Whittmore & Knafl Torraco	To review, critique, and synthesize literature related to the influences and perceptions that impact the delivery of new graduate nurse transition to practice programs	Two Screening questions were asked by the researchers about which articles would be utilized	Healthcare organizations are not delivering reliable, dependable, or value worthy support to new graduate nurses Preemptive psychosocial self-support methods may need to be taught prior to graduation	V / A

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Nweke et al., (2021)	*N*=300 / nursing students/ Nigeria	Descriptive cross-sectional study	Not noted	Not noted	To assess readiness to resume clinical practice amid COVID-19 virus concerns	Data was collected via an online survey using google forms sent via whatsup app and email. Random sampling from 5 institutions. Convenience sampling for nursing student	61.7% of respondents were ready to return to clinical practice. Only 30.7% of respondents were ready to return to clinical practice with COVID-19 patients.	II / B
Patterson et al., (2017)	*N*=28/ graduate nurses/ Australia	cross-sectional survey/ Work Readiness Scale (WRS)/ validity and reliability not addressed	Not noted	Not noted	To examine the perception of work readiness of a new graduate nurse who participated in one of three training models.	Self-administered survey	New nurse graduates who spent consistent pre-graduate time working in the same organization as their professional roles, were better able to enter these jobs with a broader level of incorporated knowledge and performed better within the organization	III / C

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Peng et al., (2022)	*N*= 13 / studies/ China	Qualitative meta-synthesis	Not noted	Not noted	To identify, appraise and synthesize data from qualitative studies the stress experience and coping styles of new nurses during residency	Systematic review of eleven databases to identify relevant articles	Positive coping mechanisms benefit the new graduate nurse in the transition to practice More strategies are needed to enhance the external effects on nurse transition	III / B
Powers et al., (2021)	*N*=116 /nurse faculty/ North Carolina, United States	Convenience sample	Not noted	Not noted	Comparison of faculty perception of work readiness between students graduating pre-COVID-19 and during COVID-19	Convenience sampling of nursing faculty in North Carolina nursing programs. Email and online survey link sent to eligible participants	A statistically significant decrease in faculty perception of new graduate nurse readiness was noted. Administration of medication was a significantly decreased readiness score Study results will help to prepare and plan for future disruptions of clinical learning	V / B
Ragsdale & Schuessler (2021)	*N*=48 /published papers on simulation and senior practicum/ United States	Integrative review	What impact does senior practicum and simulation have on graduating senior nursing student readiness for practice (GSNS-RFP)	Whittmore & Knafl	To explore the influence of simulation and practicum on graduating nurses' readiness for practice	A literature was performed utilizing five databases and limited to full text, peer reviewed, English, abstract provided articles	Complex multi-patient simulations hold greater value in preparing student nurses for practice Practicums that encourage clinical reasoning must be identified by educators Nursing students find value in opportunities to practice basic nursing tasks and psychomotor skills	V / B

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
Rogers et al., (2021)	*N*=20/ Studies or articles/ international, researcher from Australia	Integrative review	Not noted	Whittmore & Knafl	To synthesize evidence regarding strategies to assist in the transition of new graduate transition to practice	A systematic review of literature was performed utilizing 10 databases and included 3 grey literature databases	Amongst the differing methodologies of delivering TTP program common themes such as clinical support, study time, additional training time and multi-unit rotation were prevalent. Delivery of training is subject to the locality thus allowing for differences	V / B
Shatto et al., (2022)	*N*=40 / peer-reviewed articles/ United States	Systematic Review	Not noted	Not noted	To determine whether there is evidence of the objective assessment of learning in the senior year that aligns with the new AACN domains.	Systematic review of literature performed	Six of the 10 domains were addressed in the literature. Many studies used investigator-developed tools with no validity or reliability.	I / B
Ulmen et al., (2022)	*N*=124/ baccalaureate RN grads with at least one semester interrupted by the pandemic/ *US*, Canada, New Zealand, Australia, UK	Descriptive correlational study - mix methods/ Casey Fink Graduate Nurse Experience Survey plus 3 additional administered/ validity and reliability of Casey Fink not mentioned	Not noted	Not noted	To examine the effect of virtual clinical alternatives during the COVID-19 pandemic and its effect on new graduate nurse readiness	Homogenous purposive sampling Data were uploaded into and analyzed by SPSS v25	Virtual clinical replacement was shown to be valuable and manageable replacement for in person clinical learning needs during the pandemic Virtual clinical replacement is not for everyone Further research to compare virtual exposed	III / B

Author, Year	Sample/ population/ setting	Design/ Instrument/ Psychometrics	Research Question /Hypothesis	Theoretical Framework	Purpose	Procedure for Data Collection	Results	Evidence Level/Quality
							and unexposed groups is needed Other methods of virtual teaching must be evaluated.	
Xu et al., (2021)	*N*=71/ *n*=38 new graduate RNs assigned to the intervention group *n*=41 NGRNs assigned to the control group/ China	Quasi-experiment	Not noted	Not noted	To examine the effect of group counseling on transition shock in new graduate RNs	Random assignment of intervention and control group Two-part questionnaire administered Statistical analysis performed via SPSS v 22.0	Total overall scores of Transition Shock Scale decreases noted for those who were counseled thus. Counseling alleviates transition shock	II / B

Note. The table included studies in the category of new graduate nurse readiness for practice strategies. This resulted from research of the literature at the time when there was very little known about the long-term effects of the COVID-19 pandemic on nursing students' transition to professional practice as it relates to soft skills acquisition.

Appendix E

Participant Invitation Letter

Dear Nursing Student:

We are conducting a study of nursing students enrolled in an associate degree program. We are interested in learning about your perception of confidence and readiness to enter the nursing profession.

The purpose of this letter is to ask for your participation in this study. If you agree to participate, please complete the attached survey, Modified Casey-Fink Readiness for Practice Survey ©2008. This survey should take approximately 10 minutes to complete. All your answers will be kept completely confidential. The survey results will have no identifying information on it and no Individual identities will be used in any reports or publications that may result from this study. The survey asks for your thoughts on being a nursing student in your current program. There is no financial benefit to you for participating in this study and there will be no reimbursement provided. There are no financial costs to you because of taking part in the study. The survey results may be beneficial in aiding schools and colleges of nursing better prepare for future nursing students.

Your completion of this survey is an indication of your participation in this project. You may withdraw your participation from this project at any time. If you have questions about this project please contact the investigators (Torri Crosby-Martin, 708-262-3668 or email at tcrosbymartin@student.govst.edu); Dr. Shirley Spencer, 708-534-2000 or email at sspencer@govst.edu. If you have questions or concerns regarding the study itself, you may also contact the GSU Institutional Review Board at 708-543-4392 or email at irb@govst.edu

Thank you in advance,

Torri Crosby-Martin MSN, RN, C-EFM (Student Investigator)

Shirley Spencer (Primary Investigator)

Appendix F

Participant Code_____ **Date_____**

Pre-Education/Activity Survey: Communication

This survey is confidential. Your response to these questions will be used solely for statistical purposes. This survey is voluntary. You have the right to refuse to answer any and all questions. There are no risks involved in completing this survey. By completing this survey, you are giving informed consent to participate in the collection of this data. Data collected will be combined and reports will only cite aggregate figures.

The purpose of this pre-activity survey is to assess your level of agreement as it relates to your knowledge and perception of communication within a healthcare team. Participation may help increase nurse retention and satisfaction while addressing safety concerns regarding medication errors, failure to recognize urgency and/or rescue which leads to poor patient outcomes.

☐Project Participant ☐ Non-Participant
(mark as applicable)

COMMUNICATION	*Mark one response per statement:*			
Please rate your level of agreement with each of the following statements:	Strongly Disagree	Disagree	Agree	Strongly Agree
1. I am confident in the knowledge of my role in communication within the healthcare team.	○	○	○	○
2. I am confident in understanding the role of other health care members in communication within the health care team	○	○	○	○
3. I am confident in the ability to effectively communicate with patients and their family members.	○	○	○	○
4. I am confident in the ability to communicate with members of the interprofessional team (physicians, nurses, social work, etc).	○	○	○	○

(Adapted from www.aacnnursing.org/essentials)

Legend

1-Strongly Disagree

2-Disagree

3-Agree

4-Strongly Agree

Thank you for your participation in this activity.

Appendix G

PROGRAM LOGIC MODEL

NAME OF PROGRAM/PROJECT:
Design and Implementation of an Evidence-Based Pilot Program to Support New Graduate Nurses Transition to Practice: Application of Soft Skills Education in an Associate Degree Program

SITUATION:
New nurse graduates continue to have difficulty adapting to professional practice since the COVID-19 pandemic

PRIORITIES:
To improve practice readiness, confidence, and competence in new graduate nurses through soft skills educational activities

INPUTS	OUTPUTS		OUTCOMES		
	Activities	**Participants**	**Short-term**	**Medium-term**	**Long-term**
*Student- Study Participants will complete educational activities. *College instructors and/or student project manager taught and implemented educational activities *Simulation lab supervisor- assisted in preparing for final simulation *School learning management system modules- was used for additional education and learning assessments	*Project recruitment *5-week didactic, hands-on and/ or simulated educational sessions. Timeline: March 2024 through April 2024 incorporating time for spring break.	First and Second year associate degree nursing students at a local community college will complete 5-weeks didactic, hands-on, and simulated educational sessions and will complete pre- and post-test surveys throughout via bi-weekly assessment	Basic understanding of what soft skills is and why they are desired for new graduates Timeline: Weeks 1,2,3,4, & 5 within overall program (based upon school calendar & dependent on spring break)	Participants demonstrate practice readiness for successful application of 5 soft skills through Casey Fink Readiness for Practice Survey© 2008 results. Timeline: End of 6 week or early week 7	Participants transition to practice and demonstrate practice confidence via Casey-Fink Graduate Nurse Experience Survey© (CFGNE) (revised 2023) results. -New nurses will report practice readiness. -The partner hospital will report on improved retention in new graduate staff. Timeline: 6- and 12-months post-associate degree new graduate employment at participant hospital.

PROGRAM LOGIC MODEL

ASSUMPTIONS	EXTERNAL FACTORS
1. Nursing transition to practice will be seamless 2. New graduate nurses will demonstrate improved practice readiness	1. Participant absences 2. Participant fails a class or leaves nursing program 3. Participant withdraws from project

Evaluation PLAN:
Data collection from pre- and post-implementation surveys. The CFGNE survey administered at six months and one year post employment will be analyzed utilizing SPSS statistical analysis software

APPENDIX H

Applicant: Terri Crosby Martin

Subject: IRB Application Status

Date: February 7, 2024

Dear Ms. Crosby Martin,

The IRB committee at South Suburban College has reviewed your application for the proposed participant study for your graduate work. Based on the information provided and included materials, the committee approves your research on this topic. Congratulations.

The committee recommends that you retain all records of your research proposal. Continuation of this approval is dependent on you submitting additional review applications or continuation forms should the scope of your project exceed one year past the IRB approval date. Please contact the IRB committee at that time to discuss processes for continuation of this IRB approval. At that time, a subsequent review will evaluate your research protocols to ensure future compliance. Good luck in your research.

Sincerely,

Kevin M. Riordan

Dr. Kevin M. Riordan, Ed.D.

Chair, Institutional Review Board

South Suburban College

APPENDIX I

Governors State
UNIVERSITY

Institutional Review Board
Room G353
1 University Parkway
University Park, IL 60484
www.govst.edu/irb

Apr 24, 2024 1:14:37 PM CDT

Shirley Spencer, Torri Crosby-Martin
Nursing

Re: Modification submission - IRB-FY2024-56 Design and Implementation of an Evidence-Based Pilot Program to Address New Graduate Nurses Transition to Professional Practice: Application of Soft Skills Education in an Associate Degree Program

Dear Dr. Shirley Spencer, Torri Crosby-Martin:

The Governors State University Institutional Review Board has approved the Modification submission of your research protocol titled, "Design and Implementation of an Evidence-Based Pilot Program to Address New Graduate Nurses Transition to Professional Practice: Application of Soft Skills Education in an Associate Degree Program."

At the end of the year, if your research is completed, please complete a Project Closure submission. If you intend to continue the project or collect data using human subjects, you must complete a **Renewal Submission**. If you would like to make any changes to research personnel or to the way that this research is conducted, you must completed a **Modification submission** and wait for IRB approval to enact any changes.

Please be advised that you may only distribute the **GSU IRB stamped "approved"** consent forms and recruitment materials to all participants. Your stamped consent form can be found under the "Attachments" tab on the "Study Details" page.

Please include the assigned IRB project number, PI name, and exact title of your project in any correspondence about this project.

Decision: Exempt

Selected Category: (4) Secondary research for which consent is not required: Secondary research uses of identifiable private information or identifiable biospecimens, if at least one of the following criteria is met:(ii) Information, which may include information about biospecimens, is recorded by the investigator in such a manner that the identity of the human subjects cannot readily be ascertained directly or through identifiers linked to the subjects, the investigator does not contact the subjects, and the investigator will not re-identify subjects;

Sincerely,
Governors State University Institutional Review Board
irb@govst.edu

APPENDIX J

COLLABORATIVE INSTITUTIONAL TRAINING INITIATIVE (CITI PROGRAM)

COMPLETION REPORT - PART 1 OF 2
COURSEWORK REQUIREMENTS*

* NOTE: Scores on this Requirements Report reflect quiz completions at the time all requirements for the course were met. See list below for details. See separate Transcript Report for more recent quiz scores, including those on optional (supplemental) course elements.

- **Name:** Torri Crosby-Martin (ID: 11008918)
- **Institution Affiliation:** Governors State University (ID: 867)
- **Institution Email:** tcrosbymartin@student.govst.edu
- **Phone:** 7089154600

- **Curriculum Group:** Human Research
- **Course Learner Group:** IRB Social/Behavioral Research
- **Stage:** Stage 1 - Basic Course

- **Record ID:** 53746705
- **Completion Date:** 31-Jan-2023
- **Expiration Date:** 31-Jan-2026
- **Minimum Passing:** 80
- **Reported Score*:** 91

REQUIRED AND ELECTIVE MODULES ONLY	DATE COMPLETED	SCORE
Belmont Report and Its Principles (ID: 1127)	18-Jan-2023	3/3 (100%)
Students in Research (ID: 1321)	18-Jan-2023	5/5 (100%)
History and Ethical Principles - SBE (ID: 490)	19-Jan-2023	5/5 (100%)
Defining Research with Human Subjects - SBE (ID: 491)	30-Jan-2023	4/5 (80%)
The Federal Regulations - SBE (ID: 502)	30-Jan-2023	5/5 (100%)
Assessing Risk - SBE (ID: 503)	30-Jan-2023	4/5 (80%)
Informed Consent - SBE (ID: 504)	30-Jan-2023	5/5 (100%)
Privacy and Confidentiality - SBE (ID: 505)	30-Jan-2023	4/5 (80%)
Research with Prisoners - SBE (ID: 506)	30-Jan-2023	5/5 (100%)
Research with Children - SBE (ID: 507)	30-Jan-2023	4/5 (80%)
Research in Public Elementary and Secondary Schools - SBE (ID: 508)	30-Jan-2023	5/5 (100%)
Internet-Based Research - SBE (ID: 510)	31-Jan-2023	4/5 (80%)
Governors State University Module (ID: 1638)	31-Jan-2023	No Quiz

For this Report to be valid, the learner identified above must have had a valid affiliation with the CITI Program subscribing institution identified above or have been a paid Independent Learner.

Verify at: www.citiprogram.org/verify/?kb2f1165e-db70-4989-a08e-2740986fb50f-53746705

Collaborative Institutional Training Initiative (CITI Program)
101 NE 3rd Avenue
Suite 320
Fort Lauderdale, FL 33301 US

Email: support@citiprogram.org
Phone: 888-529-5929
Web: https://www.citiprogram.org

APPENDIX K

COLLABORATIVE INSTITUTIONAL TRAINING INITIATIVE (CITI PROGRAM)

COMPLETION REPORT - PART 2 OF 2
COURSEWORK TRANSCRIPT**

** NOTE: Scores on this Transcript Report reflect the most current quiz completions, including quizzes on optional (supplemental) elements of the course. See list below for details. See separate Requirements Report for the reported scores at the time all requirements for the course were met.

- **Name:** Torri Crosby-Martin (ID: 11008918)
- **Institution Affiliation:** Governors State University (ID: 867)
- **Institution Email:** tcrosbymartin@student.govst.edu
- **Phone:** 7089154600

- **Curriculum Group:** Human Research
- **Course Learner Group:** IRB Social/Behavioral Research
- **Stage:** Stage 1 - Basic Course

- **Record ID:** 53746705
- **Report Date:** 24-Apr-2023
- **Current Score**:** 91

REQUIRED, ELECTIVE, AND SUPPLEMENTAL MODULES	MOST RECENT	SCORE
Students in Research (ID: 1321)	18-Jan-2023	5/5 (100%)
Defining Research with Human Subjects - SBE (ID: 491)	30-Jan-2023	4/5 (80%)
The Federal Regulations - SBE (ID: 502)	30-Jan-2023	5/5 (100%)
Belmont Report and Its Principles (ID: 1127)	18-Jan-2023	3/3 (100%)
Assessing Risk - SBE (ID: 503)	30-Jan-2023	4/5 (80%)
Informed Consent - SBE (ID: 504)	30-Jan-2023	5/5 (100%)
Privacy and Confidentiality - SBE (ID: 505)	30-Jan-2023	4/5 (80%)
Research with Prisoners - SBE (ID: 506)	30-Jan-2023	5/5 (100%)
Research with Children - SBE (ID: 507)	30-Jan-2023	4/5 (80%)
Research in Public Elementary and Secondary Schools - SBE (ID: 508)	30-Jan-2023	5/5 (100%)
Internet-Based Research - SBE (ID: 510)	31-Jan-2023	4/5 (80%)
History and Ethical Principles - SBE (ID: 490)	19-Jan-2023	5/5 (100%)
Governors State University Module (ID: 1638)	31-Jan-2023	No Quiz

For this Report to be valid, the learner identified above must have had a valid affiliation with the CITI Program subscribing institution identified above or have been a paid Independent Learner.

Verify at: www.citiprogram.org/verify/?kb2f1165e-db70-4989-a08e-2740986fb50f-53746705

Collaborative Institutional Training Initiative (CITI Program)
101 NE 3rd Avenue
Suite 320
Fort Lauderdale, FL 33301 US

Email: support@citiprogram.org
Phone: 888-529-5929
Web: https://www.citiprogram.org

APPENDIX L

Permission to Use Duchscher Model

Model Permission

From Crosby Martin, Torri (Student) <tcrosbymartin@govst.edu>
Date Sun 3/9/2025 5:43 PM
To jduchscher@tru.ca <jduchscher@tru.ca>

Good Sunday Evening, Dr. Duchscher,

My name is Torri Crosby-Martin, and I am writing to ask for permission to use the copyrighted image of your Theory of Transition Shock model in my doctoral capstone project paper. I reference and cite your Transition Theory model along with Dr. Benner's Novice-to-Expert Model to support my project to facilitate soft skills development in new graduate nurses since the Covid-19 pandemic.

I am also required to submit my project as an article for publication and may need to ask for permission at a later date.

Thank you in advance and I look forward to your response.

Torri Crosby-Martin MSN, RN, C-EFM (DNP Candidate)

Re: Model Permission

From Judy Duchscher <jduchscher@tru.ca>
Date Tue 3/11/2025 12:01 PM
To Crosby Martin, Torri (Student) <tcrosbymartin@govst.edu>

Absolutely you have my permission. Let me know if you need images.

Kukwstsetsemc (Thank-You),
Judy

Dr. Judy Boychuk Duchscher (she/her) RN, BScN, MN, PhD
Adjunct Professor, School of Nursing
Nursing & Population Health Building
Room 374
Thompson Rivers University
805 TRU Way
V2C0C8
Kamloops, British Columbia
jduchscher@tru.ca
~~~
Founder/CEO - *Nursing The Future*
https://nursingthefuture.ca
newgraduates@nursingthefuture.ca
306-716-3286 (Cell)
~~~
Principal Investigator
Canadian Association of Schools of Nursing (CASN)
Nurse Residency Program
2023-2027

APPENDIX M

Abbreviations and Acronyms

AACN	American Association of Colleges of Nursing
CDC	Centers for Disease Control Disease and Prevention
CFRPS	Casey-Fink Readiness for Practice Survey
CFGNE	Casey Fink Graduate Nurse Experience Survey
CINAHL	Cumulative Index to Nursing and Allied Health Literature
IRB	Institutional Review Board
PRISMA	Preferred Reporting Items for Systematic Reviews and Meta-Analyses
SWOT	Strengths, Weaknesses, Opportunities, Threats